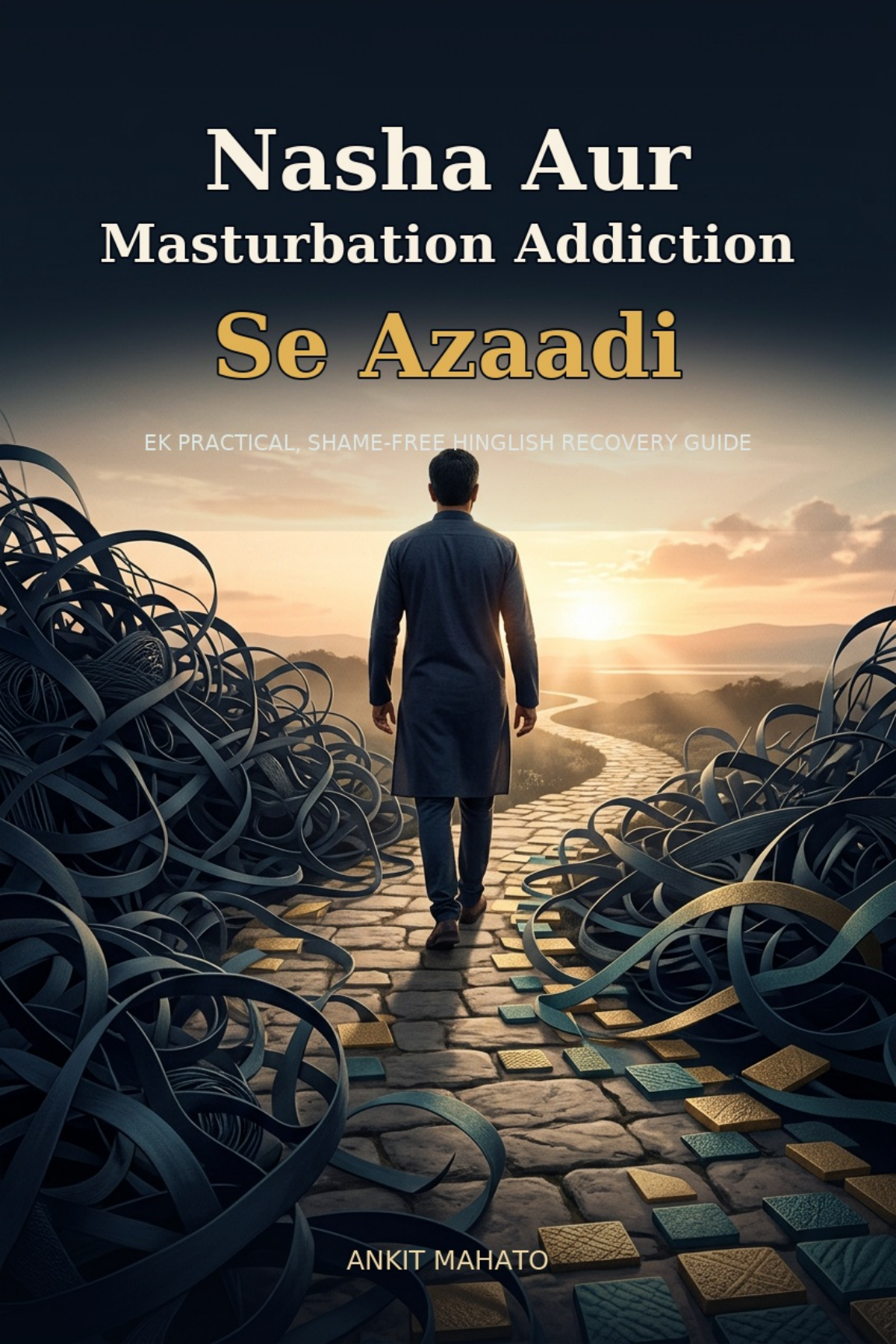
Nasha Aur Masturbation Addiction Se Azaadi

EK PRACTICAL, SHAME-FREE HINGLISH RECOVERY GUIDE



ANKIT MAHATO

AN ORIGINAL HINGLISH RECOVERY BOOK

Nasha Aur Masturbation Addiction Se Azaadi

Ek practical, shame-free, evidence-aware recovery guide

Ankit Mahato

First edition • 1 November 2026

Book ka promise
Yahan koi moral lecture, miracle cure, dopamine-reset myth ya semen-retention claim nahi milega. Milega clear science, safe treatment guidance, practical skills, 30-day workbook aur honest recovery planning.

Substance-use recovery • Compulsive masturbation/porn patterns • Consent • Professional help • Lapse recovery

READER'S GUIDE

Important Medical & Safety Note

Is page ko skip mat karo — especially agar substance use involved hai.

Yeh book educational self-help aur recovery-planning resource hai. Yeh medical diagnosis, personalised prescription, psychotherapy, emergency service, legal advice ya qualified treatment ka substitute nahi hai. Substance-use aur sexual-behaviour concerns ka assessment individual history, health, medicines, mental state, safety aur context ke bina accurate nahi hota.

Heavy prolonged alcohol use, benzodiazepines/other CNS depressants, mixed-substance use, previous severe withdrawal, pregnancy, serious medical illness, or uncertain withdrawal risk mein abrupt cessation ya self-made taper dangerous ho sakta hai. Qualified medical guidance lo. Seizure, hallucinations, severe confusion, unresponsiveness, slow/irregular breathing, blue lips, chest pain, serious injury, suicidal thoughts, violence risk ya inability to stay safe mein India mein 112 / nearest emergency service use karo.

Masturbation ya high sexual desire apne aap mein disorder nahi hai. Book ka focus persistent impaired control, significant distress/impairment, health, consent aur real-life consequences par hai. Distress jo entirely moral disapproval se aata hai, alone CSBD diagnosis establish nahi karta. Kisi bhi medication ko start, stop, restart, double ya taper karne se pehle qualified prescriber se guidance lo.

Book mein named examples fictional composites hain. Koi real patient story, testimonial, guaranteed outcome ya cure claim nahi hai. Research developing hai, especially CSBD/PPU treatment mein; promising findings ko certainty ki tarah present nahi kiya gaya. Helpline details official sources se 1 November 2026 ko check ki gayi hain; availability/services badal sakte hain, isliye current official information verify karo.

> **Immediate emergency**
> India mein 112 ya nearest emergency service. Seizure, unresponsiveness, breathing problem, severe confusion/hallucinations, chest pain, serious injury, suicidal thoughts ya violence risk mein self-help exercises ka wait mat karo.

READER'S GUIDE

How To Use This Book

Linear padho ya apni current need se start karo — safety chapters pehle.

Pehle safety, phir exercises

Agar withdrawal, overdose, severe mental-health symptoms, injury, non-consensual risk ya immediate danger possible hai, toh relevant safety/professional chapters aur urgent help ko priority do. Workbook complete karne ke liye care delay mat karo.

Apna primary track choose karo

Track S substance-use recovery ke liye, Track M compulsive masturbation/porn pattern ke liye. Dono involved hon toh highest safety risk pehle. Dono ko same day perfect karne ka pressure nahi.

Ek baar mein ek useful action

Har chapter ka exercise optional aur adaptable hai. Short notes enough hain. Missed day double karke punish mat karo. Professional plan ke against koi workbook instruction follow mat karo.

Privacy ko design mein rakho

Notes mein graphic sexual detail, illegal procurement information, passwords ya another person ki private data likhne ki zaroorat nahi. Shared device/home mein safe storage choose karo. Unsafe/controlling environment mein specialist safety advice lo.

Suggested reading routes

- Substance use involved: Chapters 4–10 pehle, phir shared skills and workbook.
- Masturbation/porn concern: Chapters 1–3, 11–14, phir relationships and workbook.
- Recent lapse: Chapter 15 and safety plan; shame spiral mein poora book start se padhna mandatory nahi.
- Family/support person: Chapters 4–5, 10, 16 and reusable support agreement.

READER'S GUIDE

Quick Safety Directory

Numbers ko phone mein save karo. Current availability official channel se verify karo.

Immediate emergency — India

112

Seizure, unresponsiveness, breathing problem, severe confusion, serious injury, immediate self-harm/violence danger or other urgent police/fire/medical emergency.

Tele-MANAS

14416 / 1800-891-4416

24×7 mental-health support, counselling and referral. Immediate physical danger/overdose mein emergency care priority.

National Drug-de-Addiction Helpline

14446

Substance-use concern mein counselling, treatment/service navigation and NAPDDR-related support.

National Tobacco Quitline

1800-11-2356

Tobacco cessation counselling and quitting support.

Emergency vs craving
Craving mein pause/support/coping plan. Emergency signs mein 112/emergency care. Withdrawal ko willpower challenge na banao.

Contents

Clickable entries • PDF bookmarks mein detailed sections bhi available hain

Nasha Aur MasturbationAddiction Se Azaadi 2

Important Medical & Safety Note 3

How To Use This Book 4

Quick Safety Directory 5

Contents 6

Pehle Sach Ko Samjho 10

Addiction, Habit Ya Sirf Ek Routine? 11

Masturbation: Normal Behaviour Kab, Compulsion Kab? 16

Dopamine, Reward Aur Shame Ka Loop 21

Safety Aur Apna Recovery Map 26

Withdrawal Safety: Kab Home Plan Nahi Chahiye 27

Baseline, Assessment Aur Professional Help 33

Triggers Ko Design Karna 38

Trigger Chain Ka X-Ray 39

Environment Design: Access Ko Difficult, Support Ko Easy 43

Nasha Chhodne Ka Practical Plan 48

Safe Quit Plan: Date Se Pehle Ka Kaam 49

Substance-Specific Guide 54

Treatment Ko Apna Ally Banao 60

Masturbation / Porn Compulsion Se Choice Wapas 65

Compulsion Ka Personal Recovery Plan 66

Healthy Sexuality, Consent Aur Relationships 71

Urge Aur Emotion Toolkit 76

Craving Toolbox: Pause Se Next Action Tak 77

Stress, Loneliness, Sleep Aur Mental Health 82

Lapse, Repair Aur Support 87

Lapse Ke Baad: First Hour Se Next Week Tak 88

Support System Aur Real-Life Repair 92

Recovery Ko Life Mein Convert Karo 97

Ninety-Day Rhythm Aur Long-Term Maintenance 98

30-Day Workbook Kaise Use Karein 102

Your 30-Day Recovery Workbook 106

Baseline Without Judgement 107

Safety Card 108

One Honest Support Request 109

Trigger Chain Mapping 110

Values-Based Goal 111

Access Aur Friction 112
Week One Review 113
Permission Thoughts 114
Three If–Then Plans 115
Grounding Practice 116
Delay With A Real Action 117
Alternative Relief Menu 118
Gentle Movement 119
Week Two Review 120
Minimum Viable Day 121
Sleep Boundary 122
Connection Ladder 123
Boredom Aur Healthy Reward 124
Consent Aur Conversation 125
Practical Repair 126
Week Three Review 127
High-Risk Event Plan 128
Lapse Plan Drill 129
Shame Se Responsibility 130
Mental Health Check 131
Support Boundaries 132
Meaning Beyond Recovery 133

Professional Plan Review 134

Green, Amber, Red 135

Review Aur Next Thirty Days 136

Reusable Worksheets 137

Trigger Chain Map 138

Coping Card 139

Clinician One-Page Brief 140

Lapse Functional Review 141

Weekly KEEP–CHANGE–ASK 142

Support Agreement 143

Relationship Repair Page 144

Next Ninety Days 145

Glossary 146

Verified Help Directory — India 149

Research Notes & References 151

Azaadi Ka MatlabChoice Ka Wapas Aana 154

PART 01



Pehle Sach Ko Samjho

Labels nahi — clarity, safety aur self-respect se shuruaat

Recovery ka pehla step khud ko ‘badnaam’ karna nahi, balki pattern ko saaf dekhna hai. Jab hum habit, dependence, addiction, high desire aur compulsion ko ek hi word mein mila dete hain, toh plan bhi galat ban jaata hai. Yeh part tumhe woh clear lens dega jiske bina baaki exercises adhoori reh jaati hain.

IN THIS PART

01 Addiction, Habit Ya Sirf Ek Routine?
Ek single act nahi, uske control aur consequences ko samjho

02 Masturbation: Normal Behaviour Kab, Compulsion Kab?
Frequency se nahi — control, impact aur context se

03 Dopamine, Reward Aur Shame Ka Loop
Brain ko villain nahi, learning system ki tarah samjho

CHAPTER 01

Addiction, Habit Ya Sirf Ek Routine?

Ek single act nahi, uske control aur consequences ko samjho

RESEARCH TRAIL R10 · R11 · R12 · R13 · R16

Ek pattern, teen basic links

Habit ko simple language mein samjho toh teen links milte hain: ek cue ya trigger, uske baad ka behaviour, aur phir koi immediate result. Stress hua, phone khola, thodi der relief mila. Friends ke saath baitha, substance offer hua, awkwardness kam hui. Brain ko woh sequence yaad reh jaata hai, kyunki usne koi discomfort turant change kiya. Is learning ka matlab yeh nahi ki tum jaan-boojhkar apni life kharab karna chahte ho. Matlab sirf itna hai ki ek short-term solution repeat hote-hote automatic lagne laga hai.

Lekin har automatic habit addiction nahi hoti. Chabi rakhna, brush karna, ya raat ko phone check karna annoying ho sakta hai, phir bhi usmein severe loss of control ya serious harm na ho. Addiction ya compulsive pattern mein behaviour ke peechhe ka pressure, repeated unsuccessful control attempts aur life par impact zyada central hote hain. Isliye hum sirf ‘kitni baar kiya?’ nahi poochhenge. Hum poochhenge: ‘Kya main choose kar pa raha hoon? Kya nuksan ke baad bhi same cycle repeat ho raha hai? Kya yeh meri responsibilities, health ya relationships ko squeeze kar raha hai?’

- Cue: kya hua, kahan tha, kis feeling mein tha?
- Behaviour: maine exactly kya kiya, bina detail mein jalebis ya judge banaye.
- Immediate result: kya relief mila — aur uske baad kya cost aayi?

Control ek switch nahi, spectrum hai

Log aksar do hi categories sochte hain: ‘control hai’ ya ‘addict hoon’. Real life zyada nuanced hai. Kabhi tum easily ruk jaate ho, kabhi specific mood, place ya people ke saath rukna bahut mushkil ho jaata hai. Kisi din ek plan follow hota hai, kisi din exhaustion ke baad nahi. Is spectrum ko dekhna useful hai kyunki treatment wahi se shuru hota hai jahan control sabse zyada tootta hai. Agar har situation ko same severity maan loge, toh plan ya toh overreaction hoga ya under-preparation.

Ek incident se apni identity mat banao. Uske baad bhi agar tum ruk kar next safe step le sakte ho, woh control ka evidence hai. Wahi dusri taraf, agar baar-baar ‘bas aaj last time’ kehkar boundaries toot rahi hain, aur tumhe lagta hai ki decision tumhare haath mein hi nahi rehta, toh use halka na samjho. Dono extremes — ‘sab kuch normal hai’ aur ‘main hopeless hoon’ — recovery ko block karte hain. Useful stance hai honest curiosity: pattern kitna persistent hai, kis context mein badhta hai, aur kaunse supports control ko wapas strengthen kar sakte hain?

> **Try this**
> Apne last three episodes ko 0–10 nahi, teen labels mein likho: ‘choice clear thi’, ‘choice mushkil thi’, ‘choice almost automatic lagi’. Ab dekho kya third label ke common triggers hain. Yeh clinical score nahi, sirf observation tool hai.

Dependence aur addiction same word nahi hain

Physical dependence ka matlab body ka kisi substance ke adjust ho jaana hai; use kam ya band karne par withdrawal aa sakta hai. Yeh kuch prescribed medicines ke saath bhi ho sakta hai, jab patient medicine exactly doctor ke instructions mein le raha ho. Addiction ya substance-use disorder mein impaired control, compulsive use aur harmful consequences central hote hain. Isliye sirf withdrawal ka hona ya tolerance ka badhna, akela, poori clinical picture nahi batata. Aur sirf ‘main doctor ki medicine leta hoon’ kehna bhi risk ko rule out nahi karta agar use prescribed way se badh-chadh raha ho.

Yeh distinction shame kam karta hai aur safety badhata hai. Agar dependence possible hai, toh abrupt stopping dangerous ho sakti hai —

especially alcohol aur sedatives mein. Agar dependence nahi bhi hai, phir bhi compulsive behaviour life ko damage kar sakta hai. Is book mein ‘nasha’ broad user-language hai: tobacco/nicotine, alcohol, cannabis, opioids, sedatives, stimulants, inhalants ya mixed use. Har substance ka risk aur treatment same nahi hai. Jo plan tobacco ke liye sensible hai, woh alcohol withdrawal ke liye unsafe ho sakta hai.

> **Important**
> Prescription medicine ko apne aap start, stop, double ya taper mat karo. Agar use badh gaya hai, early refill chahiye, ya prescribed dose ke bina discomfort hota hai, toh prescriber ko exact baat batao. Doctor ko sach batana punishment ka reason nahi, safe treatment ka input hai.

Impact ko measure karo, insult ko nahi

Self-assessment ka sabse useful question ‘Main kitna weak hoon?’ nahi, ‘Pattern kis area mein cost de raha hai?’ hai. Shayad sleep late ho rahi hai aur morning class miss hoti hai. Shayad money unexplained jagah ja raha hai. Partner ke saath baat karte hue distraction, irritation ya secrecy badh rahi hai. Kabhi physical symptoms hain — cough, injuries, pain, appetite change, withdrawal — aur kabhi social impact zyada visible hai. Har area ko alag dekhne se help ka type clear hota hai.

Ek example: agar masturbation private, consensual context mein hai, frequency vary karti hai, aur work, health, relationships par koi meaningful harm nahi, toh sirf guilt ko ‘addiction’ kehna accurate nahi hoga. Dusri situation mein person hours lose kar raha hai, repeated promises toot rahi hain, aur injury ya occupational risk ke baad bhi control nahi ho pa raha — wahan professional assessment much more relevant hai. Substance use mein bhi occasional presence aur severe disorder ko same treatment dena rational nahi. Severity ke hisaab se response badhta hai, judgement nahi.

- Health aur safety
- Sleep, study ya work
- Money aur time
- Relationships, privacy aur consent
- Mood, anxiety, shame aur isolation
- Control: stop/reduce karne ke genuine attempts kya hua?

Willpower ka myth recovery ko chhota karta hai

Willpower useful ho sakti hai, lekin woh unlimited fuel tank nahi hai. Stress, poor sleep, loneliness, easy access, withdrawal, cues aur untreated mental-health issues decision-making ko pressure dete hain. Agar plan sirf ‘aur strong ban’ hai, toh woh un real conditions ko ignore karta hai jinmein decision actually lena hai. Effective recovery plan willpower ko replace nahi karta; uske around support structure banata hai — treatment, reduced access, delay, alternative action, honest tracking aur people jo safely help kar saken.

Iska matlab responsibility khatam nahi hoti. Responsibility blame se alag hai. Blame kehta hai, ‘Mera defect hai, toh kuch nahi ho sakta.’ Responsibility kehti hai, ‘Yeh pattern real hai, aur main agla useful step choose kar sakta hoon — chahe uske liye help lena pade.’ Doctor lena weakness nahi hai, jaise fracture mein plaster lena weakness nahi. Kuch log self-help se meaningful change karte hain; kuch ko medication, structured therapy ya residential support chahiye. Need zyada hona character score nahi, care requirement hai.

> **Reframe**
> ‘Mujhe aur willpower chahiye’ ki jagah likho: ‘Mere highest-risk moment mein kaunsa extra support decision ko easier banayega?’ Is one-line question se plan practical ban jaata hai.

Seven-day observation, lifetime judgement nahi

Agar medically safe ho aur substance withdrawal risk nahi hai, toh agle seven days ka aim perfect abstinence nahi, accurate map banana ho sakta hai. Har relevant episode ya strong urge ke baad sirf four lines: situation, feeling, behaviour/urge, immediate aur later result. Masturbation/porn tracking mein explicit detail ki zaroorat nahi. Substance tracking mein type, approximate amount, time aur context clinician ke liye useful ho sakta hai, lekin kisi ko bhi detailed illegal procurement ya use technique record nahi karni chahiye.

Agar heavy alcohol use, sedative dependence, opioid use, mixed substances, previous severe withdrawal, pregnancy, serious medical condition, or suicidal thinking involved hai, toh observation ko excuse bana kar professional appointment delay mat karo. Tab tracking ka purpose appointment ke liye information collect karna hai, ‘pehle khud prove karunga phir doctor’ nahi. Safety note ke saath map banane se clinician ko pattern samajhne mein help milti hai aur tumhe bhi all-or-nothing thinking kam hoti hai.

SEVEN-DAY MAP

01 Ek private, secure note banao; agar device shared hai toh sensitive detail avoid karo.

02 Har entry mein cue, feeling, action, short-term result aur later cost likho.

03 Day seven par sirf two patterns choose karo jo sabse zyada repeat hue.

04 Unhi two patterns ke liye next chapter mein support aur safety plan banao.

CHAPTER 02

Masturbation: Normal Behaviour Kab, Compulsion Kab?

Frequency se nahi — control, impact aur context se

RESEARCH TRAIL R1 · R2 · R3 · R4 · R8 · R9 · R21 · R22 · R26

Normal variation ko disease mat banao

Masturbation ek common sexual behaviour hai. Log iski frequency ko lekar bahut vary karte hain: koi regularly, koi kabhi-kabhi, koi bilkul nahi. In differences ka apne aap mein koi universal medical cutoff nahi hota. Sirf isliye ki tumne kisi online post mein ‘normal limit’ padhi, tum us number se apna diagnosis nahi bana sakte. Better Health Channel aur ICD-11-related clinical writing dono over-pathologising se caution dete hain. Behaviour tab concern ban sakta hai jab control, safety, health, responsibilities ya relationships par meaningful negative impact aaye.

Is chapter ka shame-free hona deliberate hai. Shame accurate assessment ko distort karta hai: person ya toh normal desire ko bhi abnormal bol deta hai, ya actual loss of control ko chhupata hai. Hum dono mistakes avoid karenge. Tum masturbation ko personally choose na karo — woh bhi valid hai. Lekin ‘main abstain karna chahta hoon’ personal goal ho sakta hai; ‘jo karta hai woh weak/ganda hai’ scientific fact nahi. Goal ko apne values, comfort, relationships aur functioning se define karo, internet ke fear-based claims se nahi.

> **Core distinction**
> ‘Mujhe yeh behaviour pasand nahi’ ek preference ho sakti hai. ‘Main isse control nahi kar pa raha, harm ke baad bhi continue ho raha hai, aur life impaired ho rahi hai’ ek clinical concern ho sakta hai. Dono ko alag language dena recovery ko accurate banata hai.

Compulsion ke practical signals

ICD-11 framework mein compulsive sexual behaviour disorder ka centre persistent failure to control intense, repetitive sexual urges aur resulting behaviour hai. Extended period mein pattern responsibilities ko neglect kar sakta hai, repeated unsuccessful reduction attempts ho sakte hain, adverse consequences ke baad bhi continuation ho sakti hai, ya satisfaction kam hone ke baad bhi behaviour driven lag sakta hai. Isse marked distress ya significant impairment judta hai. Yeh simple checklist nahi jisse tum khud ko diagnose karo; qualified assessment context aur exclusions bhi dekhti hai.

Daily language mein iska matlab yeh ho sakta hai: tum decide karte ho ki kal work deadline ke baad hi, phir bhi repeated episodes deadline ko damage karte hain. Tumhe physical soreness ho, phir bhi rest nahi le paate. Relationship mein agreed boundaries baar-baar tootti hain. Ya behaviour ab pleasure bhi nahi de raha, phir bhi ‘stop nahi kar sakta’ feeling strong hai. Ek bad week ya high desire in sabke barabar nahi. Pattern, persistence, control aur impairment ko saath dekhna zaroori hai.

- Kya control ke genuine attempts repeatedly fail hue?
- Kya health, sleep, work, study, money ya relationships affected hui?
- Kya behaviour consequences ke baad bhi continue hua?
- Kya satisfaction kam hone ke baad bhi driven repetition hai?
- Kya yeh extended pattern hai, situational short phase nahi?

Guilt aur impairment ko mix mat karo

ICD-11 guidance specifically kehti hai ki distress jo entirely moral judgments ya disapproval se aata hai, woh akela CSBD criterion meet nahi karta. Is line ka practical value bahut bada hai. Agar kisi ne tumhe childhood se ‘ise karne wala insaan kharab hota hai’ sikhaya, toh normal desire ke baad bhi intense guilt ho sakta hai. Woh guilt real feeling hai, lekin uska hona alone loss of control prove nahi karta. Aise case mein fear-based abstinence promises anxiety ko badha sakte hain, jabki accurate sexual-health education aur values-based counselling zyada useful ho sakti hai.

Dusri taraf, ‘sirf guilt hai’ kehkar actual harm dismiss karna bhi galat hai. Agar work, safety, relationships ya health affected hai, toh un impacts ko

seriously lo. Helpful question hai: ‘Agar koi moral lecture mujhe na sunata, tab bhi kya yeh behaviour meri life mein concrete problems create kar raha hai?’ Answer yes ho toh us problem ko address karo. Answer no ho, lekin distress phir bhi strong hai, toh bhi support lo — bas diagnosis assume mat karo. Anxiety, shame aur relationship conflict deserve care, chahe CSBD ho ya na ho.

Two-column check
Ek column mein ‘concrete impact’ likho: missed work, injury, broken agreement, lost sleep. Doosre mein ‘judgement-based fear’: ‘log kya kahenge’, ‘main impure hoon’. Dono feelings ko respect karo, lekin plan concrete impact aur actual control par build karo.

Porn aur masturbation linked ho sakte hain, same nahi hain

Kuch log masturbate karte hain bina porn ke; kuch log porn dekhte hain bina masturbation ke. Kabhi dono ek pattern mein aate hain. Isliye plan ko exact behaviour samajhna hai. Agar primary issue late-night scrolling aur escalating content search hai, toh device aur time-based barriers useful ho sakte hain. Agar primary issue stress ke response mein sexual behaviour hai, toh emotion-regulation skills central hongi. Sirf ‘phone block kar diya’ se underlying loneliness ya anxiety address nahi hoti.

Pornography use bhi apne aap mein uniform disorder nahi hai. Problematic use mein impaired control aur significant impact relevant hain. Content ki legality, consent aur ethics bhi non-negotiable hain: kisi bhi illegal, non-consensual, exploitative ya minor-involving material ko seek, save ya share karna unacceptable hai. Agar aisa risk hai, toh immediately distance create karo aur qualified professional/legal guidance lo. Is book mein kisi bhi explicit material ko normalise ya describe nahi kiya gaya. Focus healthy choice, consent, privacy aur functioning par hai.

- Behaviour exactly kya hai — scrolling, seeking, masturbation, ya combination?
- Kaunsa device, time aur emotion entry point hai?
- Kya content aur conduct legal, consensual aur safe hai?
- Kya partner agreements ya personal boundaries affected hain?

Sudden change ko context ke bina ignore mat karo

Agar sexual urge ya behaviour achanak dramatically badh gaya hai, toh sirf self-control challenge maan lena premature hoga. Stress, anxiety, depression, loneliness aur sleep disruption coping behaviour ko affect kar sakte hain. Kuch medications, substances, neurological conditions ya mood episodes — especially mania/hypomania with marked disinhibition — bhi sexual behaviour ko change kar sakte hain. ICD-11 assessment in explanations ko consider karti hai. Iska matlab har fluctuation illness nahi; matlab sudden, severe ya out-of-character change ko qualified assessment milna chahiye.

Physical symptoms bhi separate attention deserve karte hain. Persistent pain, injury, swelling, bleeding, numbness, skin damage, urinary symptoms ya sexual-function change ke liye doctor/sexual-medicine professional se consult karo. Online forums ke diagnosis ya supplements avoid karo. Agar behaviour kisi another person ko involve karta hai, toh explicit consent aur legal boundaries essential hain. Professional help lena embarrassing lag sakta hai, lekin clinician ka role behaviour ko shame karna nahi, cause aur care options assess karna hai.

> **Escalate, don’t hide**
> Sudden personality/behaviour change, severely reduced sleep with unusually high energy, hallucinations, substance-linked disinhibition, injury, ya self-harm thoughts ho toh self-help book ko wait mat karo. Doctor, mental-health professional ya emergency support choose karo.

Goal apne values se banao, streak fear se nahi

Kuch log complete abstinence choose karte hain, kuch controlled reduction, kuch porn-free but not necessarily masturbation-free approach, aur kuch pehle sirf work/sleep/relationship damage reduce karna chahte hain. Inmein se koi universal moral ranking nahi. Goal tab useful hai jab woh specific, realistic aur safety-aligned ho. ‘Main kabhi urge feel nahi karunga’ unrealistic hai. ‘Jab urge aaye, main pehle ten-minute pause aur alternate action try karunga; repeated loss of control ho toh therapist se appointment lunga’ actionable hai.

Streak count motivation de sakta hai, lekin agar ek lapse par poori self-worth collapse ho jaaye toh counterproductive hai. Better metrics: kitni baar pause use kiya, kitni baar support liya, kitne high-risk contexts identify hue, sleep/work/relationship mein kya improvement aayi, aur lapse ke baad kitni jaldi safe reset hua. Abstinence choose karne par bhi yeh skills matter karte hain. Recovery ka aim sirf ‘zero’ dikhana nahi, aisi life jismein choice wapas ho aur behaviour tumhari values se align ho.

MERA VALUES-BASED GOAL

01 Main kis concrete impact ko reduce karna chahta hoon?
02 Mera chosen boundary kya hai, aur woh kyun meaningful hai?
03 Urge aane par mera first safe action kya hoga?
04 Kis situation mein main professional help add karunga?
05 Main progress ko streak ke alawa kaise notice karunga?

Kab self-help enough nahi hai

Agar behaviour repeatedly out of control hai, significant harm continue ho raha hai, ya attempts ke baad bhi improvement nahi aa raha, toh psychologist, psychiatrist, sexual-medicine clinician ya trained therapist se assessment lo. CBT-based approaches, motivational work, ACT, mindfulness-based skills aur relapse prevention mein emerging support hai, lekin CSBD research abhi developing hai. Koi magic exercise, pill ya guaranteed programme nahi. Medication kuch cases mein specialist consider karte hain, often off-label; khud start karna safe nahi.

Immediate help tab lo jab self-harm ya suicide thought, violence risk, non-consensual behaviour, illegal material, severe injury, acute substance effect, ya another person in danger ho. India mein mental-health support ke liye Tele-MANAS 14416 / 1800-891-4416 available hai; immediate emergency mein 112. Helpline number se diagnosis nahi hota, lekin next safe step mil sakta hai. Agar kisi aur ko risk hai, uski safety ko privacy se upar rakho aur urgent qualified help lo.

No shame in referral
Professional ke paas jaana ‘failure’ nahi. Yeh show karta hai ki tum problem ko accurately treat karna chahte ho, sirf privately struggle nahi karna.

CHAPTER 03

Dopamine, Reward Aur Shame Ka Loop

Brain ko villain nahi, learning system ki tarah samjho

RESEARCH TRAIL R1 · R2 · R3 · R4 · R10 · R11 · R12 · R13 · R14 · R16

Dopamine ko poison samajhna science nahi hai

Social media par dopamine ko aise present kiya jaata hai jaise woh har pleasure ke peechhe ek toxic chemical hai. Reality mein dopamine learning, motivation, movement aur reward-related prediction jaise multiple processes mein role karta hai. Tumhe food, music, conversation, achievement aur substances/behaviours ke cues se linked responses sab involve kar sakte hain. ‘Dopamine ho gaya’ ka matlab addiction nahi, aur ‘dopamine zero karna’ na possible hai na desirable. Recovery ka aim dopamine ko eliminate karna nahi, choice aur functioning restore karna hai.

Isliye ‘dopamine detox’ ko literal reset ki tarah present karna misleading hai. Kuch log stimulation kam karke, boring tasks ko tolerate karke aur compulsive cues se break lekar benefit feel karte hain. Woh behavioural change useful ho sakta hai, lekin uska explanation ‘brain ka dopamine flush ho gaya’ nahi hai. Better wording: ‘Main reward cues ko redesign kar raha hoon, automatic responses ko pause de raha hoon, aur meaningful activities ko rebuild kar raha hoon.’ Yeh accurate bhi hai aur actionable bhi.

> **Myth → useful wording**
> ‘Dopamine detox karna hai’ → ‘High-risk cues kam karne hain aur alternative rewards add karne hain.’ ‘Brain reset ho jayega’ → ‘Practice, treatment aur time se new patterns strengthen ho sakte hain.’

Cues prediction ko speed dete hain

Repeated pattern mein brain behaviour se pehle hi response prepare kar sakta hai. Particular room, phone icon, loneliness, argument, payday, smell, place ya friend ka message cue ban sakta hai. Cue aate hi body aur attention narrow ho sakti hai: ‘Abhi chahiye.’ Yeh feeling intense ho sakti hai, phir bhi uska matlab action compulsory nahi. Cue response ko notice karne ke liye pehle thoda observation chahiye, kyunki automatic sequence bahut fast lagti hai.

Trigger ko naam dena distance create karta hai. ‘Friday night + alone + tired + phone in bed’ ek compound cue ho sakta hai. ‘Stress + substance-using friends + cash available’ doosra. Agar tum sirf ‘mood kharab tha’ likhte ho, toh plan vague rahega. Agar exact chain dikh jaaye, toh multiple intervention points milte hain: cash access, friend conversation, bedtime routine, phone location, food, support call. NIDA substance-use recovery mein stress aur drug-linked cues ko important relapse triggers batata hai; CSBD mein bhi cue-awareness CBT-style work ka common component hai.

- Person / place / time
- Emotion / body state
- Device / money / access
- Thought jo permission deta hai
- First tiny action jo chain start karta hai

Short-term relief long-term cost ko hide karta hai

Compulsive behaviour aksar sirf pleasure ke liye nahi hota; kabhi woh anxiety, boredom, shame, anger, emptiness ya withdrawal ko temporarily reduce karta hai. Jab koi behaviour discomfort kam karta hai, brain usse solution ki tarah learn kar sakta hai — chahe relief brief ho aur baad mein cost high. Isliye ‘mazaa aata hoga, tabhi karte ho’ oversimplification hai. Kabhi person behaviour se little satisfaction bhi feel kare, phir bhi relief-seeking repeat ho.

Recovery mein alternative ko equally instant aur equally powerful banana har baar possible nahi. Walk, breathing ya call immediate numbness na

de, phir bhi useful hain kyunki woh wave ko act-out kiye bina pass karne mein help karte hain. Unka goal old behaviour ko perfect substitute karna nahi, nervous system aur decision ko enough space dena hai taaki next choice possible ho. Long-term mein sleep, relationships, meaningful work aur treatment discomfort ke drivers ko reduce karte hain, jisse relief-seeking pressure naturally kam ho sakta hai.

> **Ask the function**
> 'Yeh behaviour mujhse kya karwana chahta hai?' se better: 'Main is moment mein kis discomfort se relief chahta hoon?' Relief ka naam milte hi alternatives specific ban jaate hain.

Shame loop: 'Main kharab hoon' se 'pattern samajhna hai'

Shame behaviour ko hide kar sakta hai. Jab identity hi 'ganda', 'weak', 'hopeless' ban jaati hai, toh lapse ke baad help lene ki capacity kam hoti hai. Secrecy isolation badhati hai; isolation high-risk states ko strong kar sakta hai; phir behaviour ke baad aur shame. Self-compassion is loop ko interrupt karta hai, kyunki woh accurate reporting ko possible banata hai. Agar tumhe lagta hai ki sach bolne par tum sirf judge honge, toh journal aur therapist dono mein editing shuru ho jaati hai.

Self-compassion ka matlab excuse nahi. Excuse kehta hai, 'Kuch karne ki zaroorat nahi.' Compassion kehta hai, 'Yeh harmful pattern hai, main isse change karne ke layak hoon, aur agla safe step kya hai?' Responsibility aur dignity saath reh sakte hain. Agar kisi trusted person se baat kar rahe ho, toh pehle batao ki tumhe solution chahiye ya listening. Agar person shaming, threatening ya controlling ho, toh woh safe support nahi; qualified confidential help choose karna better ho sakta hai.

- Fact: kya hua, bina insult ke.
- Impact: kya cost ya risk aaya?
- Need: ab kis support ya action ki zaroorat hai?
- Next step: next 24 hours mein kaunsa realistic move?

Counter ke bajay learning track karo

Counter sirf ‘kitne din clean’ dikhata hai; woh nahi batata ki kaunse skills improve hue. Counter helpful ho sakta hai, lekin jab uske tootne par ‘sab khatam’ reaction aata hai, toh woh risk badha sakta hai. Uske saath ya uski jagah learning dashboard use karo: pauses tried, support calls, therapy attended, high-risk contexts avoided, alternative actions completed, sleep/work repaired. Yeh metrics behaviour change ko visible karte hain, even when perfect streak nahi hai.

Tracking bhi compulsive ho sakta hai. Har minute check, repeated body checking, constant urges rating, ya day ko multiple times score karna anxiety badha sakta hai. One brief daily review enough ho sakta hai. Substance-use treatment mein clinician monitoring useful ho sakti hai; usmein exact purpose hota hai, punishment nahi. Agar tracking khud trigger ban raha hai, toh simplify karo: morning one intention, evening one sentence, weekly professional/support review. Data ko decision ke liye use karo, self-worth calculate karne ke liye nahi.

ONE-PAGE DASHBOARD

01 Aaj ka main high-risk window kaunsi thi?
02 Maine kaunsa pause/support/action use kiya?
03 Kya maine boundary maintain ki? Agar nahi, kya safe reset kiya?
04 Kal ka one small adjustment kya hai?
05 Kya mujhe professional help add karni chahiye?

Shared skills, different evidence

Substance-use disorders aur compulsive sexual behaviour kuch surface similarities share kar sakte hain: cues, craving, impaired control, relief, consequences. Lekin unki medical risks aur evidence base same nahi. Alcohol/sedative withdrawal life-threatening ho sakta hai; masturbation compulsion mein woh specific withdrawal danger nahi hota. Opioid-use disorder ke liye medication first-line ho sakti hai; CSBD mein medication specialist-led aur often off-label hai, with limited evidence. ICD-11 CSBD ko impulse-control disorder mein rakhta hai, substance-use aur addictive behaviours ke same group mein nahi.

Isliye book shared tools — trigger mapping, CBT-style thought work, delay, support, relapse prevention — use karti hai, par unhe same guarantee ke

saath present nahi karti. Substance chapter mein medical safety central hai; compulsion chapter mein over-diagnosis avoid karna, values, consent aur functioning central hain. Research humility ka matlab kam hope nahi; matlab accurate hope. Log improve hote hain, treatment help karta hai, aur new skills learn ho sakti hain — without pretending every method works equally for everyone.

Your plan, not internet's plan
Agar koi programme dono conditions ke liye same 'magic reset' sell kare, caution. Qualified care substance, severity, health, medications, mental health, goals aur support ke hisaab se tailor hoti hai.

Apni working definition likho

Ab tak ke chapters ko ek short personal definition mein convert karo. Example: 'Mera issue sirf frequency nahi; late-night stress ke baad control lose karna aur next-day work miss karna hai. Mera goal shame-free abstinence from porn aur safer sleep routine hai, aur main therapist se assessment bhi lunga.' Substance example: 'Main daily heavy alcohol use se withdrawal risk samajhta hoon; main abruptly stop nahi karunga, doctor se supervised plan banaunga aur support person ko involve karunga.' Yeh definition future decisions ko anchor deti hai.

Definition mein identity labels se zyada behaviour aur needs rakho. 'Main addict hoon' se zyada useful hai 'Main in specific situations mein control lose karta hoon aur in supports se benefit sakta hoon.' Agar tumhe label use karna comfort deta hai, woh personal choice hai; bas label ko prognosis mat banao. Tumhari current pattern tumhari permanent identity nahi. Agla part safety aur professional mapping ko detail karega, kyunki strong plan ka first requirement safe plan hai.

WRITE FIVE LINES

01 Mera target pattern kya hai?
02 Control kaise affected hota hai?
03 Concrete impact kya hai?
04 Mera primary safety concern kya hai?
05 Kaunsa support main ab include karne ko ready hoon?

PART 02



Safety Aur Apna Recovery Map

Pehle risk ko samjho, phir speed badhao

Kuch situations mein self-help exercises useful first step hain; kuch mein medical assessment sabse urgent first step hai. Yeh part tumhe red flags, treatment options aur appointment preparation dega. Iska purpose fear create karna nahi — unsafe solo attempt se bachana hai.

IN THIS PART

04 Withdrawal Safety: Kab Home Plan Nahi Chahiye
Alcohol, sedatives, opioids aur mixed use mein different risks

05 Baseline, Assessment Aur Professional Help
Doctor/therapist ke saath better conversation, better plan

CHAPTER 04

Withdrawal Safety: Kab Home Plan Nahi Chahiye

Alcohol, sedatives, opioids aur mixed use mein different risks

RESEARCH TRAIL R13 · R14 · R15 · R16 · R17 · R19 · R20 · R21 · R22

Sabse pehla rule: abrupt stop har case mein brave nahi

Internet par ‘bas band kar do’ advice strong lagti hai, lekin heavy prolonged alcohol use ya sedative/CNS-depressant dependence mein abrupt stopping dangerous ho sakti hai. Alcohol withdrawal mein tremors, sweating, nausea, anxiety, insomnia, raised pulse/BP, seizures aur delirium tremens jaise severe complications possible hain. Benzodiazepines aur similar sedatives ki withdrawal bhi severe, potentially life-threatening ho sakti hai. Isliye in situations mein book-based taper ya cold-turkey challenge dena safe nahi.

Safe plan doctor ke saath banta hai, jismein substance, amount, duration, last use, previous withdrawals, other medicines, mental/physical health aur home support discuss hota hai. Clinician outpatient monitoring, inpatient detox, medication ya gradual supervised approach mein se kuch recommend kar sakta hai. Tumhara job apne aap dose design karna nahi; honest information dena aur prescribed plan follow karna hai. Agar access ya cost issue hai, government hospital/de-addiction service aur helpline se options poochho.

> **Red-line rule**
> Heavy alcohol use, sedative dependence, previous withdrawal seizure/DT, mixed depressant use, pregnancy, serious medical illness, ya previous severe withdrawal ho toh solo abrupt cessation attempt mat karo. Urgent qualified assessment lo.

Emergency signs ko ‘normal withdrawal’ mat samjho

Seizure, hallucinations, severe confusion, extreme agitation, fainting, new severe headache, chest pain, blue/pale lips, very slow/irregular breathing, unresponsiveness, severe vomiting with inability to keep fluids, serious injury, ya suicidal/violent behaviour — inmein immediate emergency help chahiye. India mein 112 call karo ya nearest emergency department. Agar kisi substance ke baad person react nahi kar raha, breathing abnormal hai, ya collapse hua hai, toh time waste mat karo. ‘Sula do, khud theek ho jayega’ dangerous assumption ho sakti hai.

Emergency wait karte hue person ko alone chhodna, force-feed karna, forced vomiting, cold shower, more substances dena, ya unknown home remedy dena avoid karo. Operator ke instructions follow karo. Agar person safe position mein place karna possible hai aur breathing concern hai, toh emergency dispatcher guide karega; unnecessary physical restraint mat do. Substance/medicine name, approximate amount, time, existing conditions aur current symptoms clearly batao. Information chhupana treatment delay kar sakta hai.

- Call: 112 / nearest emergency service.
- Tell: kya liya, kitna estimate, kab, aur kaunsi medicines/conditions.
- Avoid: driving the person yourself while severely impaired, if emergency transport is available.
- Stay: safe distance se observe; dispatcher instructions follow karo.
- Do not: more alcohol/drugs, random sedatives, vomiting, cold shower ya punishment.

Substance-specific safety snapshot

Alcohol aur sedatives mein medically supervised withdrawal planning especially important hai. Opioid withdrawal aksar intensely uncomfortable hota hai, aur professional treatment cravings, withdrawal aur long-term retention mein help kar sakta hai. Opioid-use disorder mein buprenorphine, methadone aur naltrexone evidence-based options hain, lekin selection/timing clinician decide karta hai. Naltrexone ko current opioid use ke context mein galat time par lena precipitated withdrawal kar sakta hai; isliye self-start nahi.

Cannabis aur stimulants ke liye behavioural therapies central hain; sleep, mood, anxiety, paranoia, agitation ya psychosis jaisi symptoms ho toh assessment chahiye. Inhalants sudden severe medical harm kar sakte hain; chest symptoms, collapse ya confusion emergency hai. Mixed substances — especially alcohol, opioids aur sedatives ka combination — breathing/overdose risk ko complicated kar sakte hain. Har additional substance assessment ka part hai, side detail nahi. ‘Bas thoda sa’ bhi clinician ko batana useful hai.

- Alcohol/sedatives: abrupt stop can be dangerous; supervised plan.
- Opioids: effective medication exists; overdose and reduced-tolerance risk matter.
- Tobacco/nicotine: behavioural support + appropriate medication options; quitline helpful.
- Cannabis/stimulants: behavioural care; severe mental/physical symptoms need assessment.
- Inhalants/mixed use: unpredictable medical risk; emergency signs taken seriously.

Appointment tak ka safe holding plan

Agar withdrawal risk possible hai lekin current emergency symptoms nahi hain, toh appointment book karke wait karna alone plan nahi hona chahiye. Same-day ya earliest medical advice lo, trusted adult ko informed rakho, driving/operating machinery avoid karo, aur substances/medicines ko apne aap add/remove mat karo. Doctor jab tak clear instruction na de, internet ka taper follow mat karo. Agar symptoms escalate hon, toh appointment date ka wait nahi — emergency care.

Holding plan mein practical environment bhi hai: easy access to unknown substances kam karo, using alone avoid karo, unsafe people/places se distance, regular food/hydration as tolerated, aur emergency numbers visible. Yeh steps overdose-proof nahi hain, lekin risk kam kar sakte hain. Agar tum already prescribed medication le rahe ho, toh prescriber ko exact current use batao. Family member ko instruction do ki shame ya physical punishment nahi karna; agitated person ko provoke karna safety worsen kar sakta hai.

SAFE HOLDING CARD

01 Doctor/service ka naam aur appointment time.
02 Trusted person ka naam, phone aur role.
03 112 aur nearest emergency department location.
04 Substances/medicines ki honest list aur last-use time.
05 Kaunse symptoms par hum wait nahi karenge?

Lapse ke baad tolerance aur overdose risk

Abstinence ya reduced use ke period ke baad body ki tolerance kam ho sakti hai, especially opioids mein. Agar person purani quantity par wapas chala jaaye, toh overdose risk badh sakta hai. Isliye lapse ke baad 'ab toh sab khatam' kehkar heavy use karna medically dangerous ho sakta hai. Safe reset ka pehla step immediate risk assess karna hai: current intoxication, breathing, consciousness, mixed substances, chest symptoms, severe agitation, ya injury? Emergency signs ho toh 112.

Non-emergency lapse mein bhi clinician ko early inform karna useful hai, particularly prescribed naltrexone, buprenorphine, methadone, disulfiram, sedatives ya other medicines involved hon. Medication ko restart/stop karne ka instruction generic nahi hona chahiye; substance, last use aur treatment history matter karte hain. 'Main phir se start karunga Monday ko' delay plan na ho. Same-day support call, safe environment aur next appointment lapse ko extended return-to-use mein convert hone se rok sakte hain.

Never chase the lapse
Ek episode ke baad punishment, more substance, double medication, starvation, excessive exercise ya risky 'proof of control' mat karo. Safety → honest information → professional/support contact → revised plan.

Jab mental health ya special situation involved ho

Suicidal thoughts, self-harm, hallucinations, severe depression, mania-like symptoms, panic with inability to stay safe, ya violence risk ho toh addiction plan ke saath urgent mental-health support chahiye. Tele-MANAS 14416 / 1800-891-4416 se support/referral mil sakta hai; immediate danger mein 112. Agar koi aur person risk mein hai, toh uski safety priority hai. Substance intoxication ke time major decisions, driving, arguments ya sexual activity involving impaired consent avoid karo.

Pregnancy, breastfeeding, under-18 age, older age, liver/kidney/heart disease, seizure history, eating disorder, chronic pain, previous severe withdrawal, ya multiple medications ho toh plan specialist input ke bina copy mat karo. In contexts mein ‘general advice’ insufficient ho sakti hai. Iska matlab recovery impossible nahi — matlab personalised care zyada important hai. Correct treatment available hai, aur early disclosure unnecessary complications se bacha sakti hai.

- Immediate danger/self-harm: 112 or emergency department.
- Mental-health support/referral: Tele-MANAS 14416 / 1800-891-4416.
- Substance treatment navigation: National Drug-de-Addiction Helpline 14446.
- Tobacco support: National Tobacco Quitline 1800-11-2356.

One-page safety plan

Ab ek page par apna safety plan likho, lekin sensitive information ko secure rakho. Usmein emergency signs, contacts, current substances/medicines, allergies, major conditions, trusted person aur nearest facility ho. Agar shared home mein privacy concern hai, toh clinician ke saath safe storage discuss karo. Plan ko phone lock-screen par detailed sexual/substance history ke bina rakho; sirf emergency contacts aur action sequence enough hai.

Safety plan ko trusted person ke saath rehearse karo: ‘Agar main unresponsive hoon, toh 112; agar main sirf craving mein hoon, toh yeh alternative contact.’ Dono situations ko same response dena useful nahi. Family/friends ko bhi clear karo ki punishment, forced detox, tying/restraining, withholding prescribed medicine, ya public shame safe

treatment nahi. Qualified, humane care hi plan hai.

MY SAFETY SEQUENCE

01 RED: emergency signs → 112 / emergency department.

02 AMBER: possible dependence or repeated loss of control → same-day professional advice.

03 GREEN: stable urge, no medical red flags → pause, support person, alternate action.

04 After any event: facts record, prescribed instructions follow, plan review.

CHAPTER 05

Baseline, Assessment Aur Professional Help

Doctor/therapist ke saath better conversation, better plan

RESEARCH TRAIL R3 · R4 · R7 · R8 · R10 · R11 · R13 · R14 · R16 · R17 · R18 · R20 · R24 · R25 · R26

Assessment ka goal label nahi, formulation hai

Good assessment sirf ‘addiction hai ya nahi’ ka yes/no nahi hoti. Clinician pattern, severity, duration, triggers, previous attempts, withdrawal risk, physical health, mental health, medications, relationships, work/study, safety aur available support samajhta hai. CSBD assessment mein high desire, moral distress, actual impaired control, other conditions aur functional impact differentiate kiye jaate hain. Substance assessment mein substance-specific risks aur treatment needs central hote hain.

Isliye one-minute online quiz ko final verdict mat samjho. Screening tool useful ho sakta hai, lekin diagnosis aur plan qualified conversation se banta hai. Tum apne notes se clinician ka kaam easy kar sakte ho: approximate amounts/frequency, last use, consequences, previous treatment, current medications, allergies, medical history, and what you want to change. Agar exact number yaad nahi, honest estimate bolo; guessing ko fact mat banao.

> **Useful opening line**
> ‘Maine pattern ko privately control karne ki koshish ki hai, ab main safe assessment chahta hoon. Mera main concern ___ hai, aur mere saath ___ bhi ho raha hai.’

Kaunsa professional kis role mein?

Physician/general practitioner initial health assessment, withdrawal risk, physical complications, medication review aur referral mein help kar sakte hain. Psychiatrist substance-use disorders, mental-health conditions, medication aur complex presentations assess karte hain. Clinical psychologist/trained psychotherapist CBT, motivational work, ACT aur relapse-prevention skills de sakte hain. Sexual-medicine specialist ya qualified sex therapist sexual-health concerns, functioning, relationships aur CSBD assessment mein useful ho sakte hain. De-addiction centres multidisciplinary support provide kar sakte hain.

India mein government hospital, medical college, district de-addiction services, NAPDDR-linked centres, NIMHANS/other addiction services aur qualified private professionals options ho sakte hain. National Drug-de-Addiction Helpline 14446 navigation ke liye use karo. Kisi centre ko choose karne se pehle humane care, qualified staff, medical support, consent, privacy, aftercare, charges aur emergency protocol poochho. Forced labour, violence, public humiliation, unlawful confinement ya miracle cure claims red flags hain.

- Credentials aur experience kya hai?
- Medical withdrawal support available hai ya referral hota hai?
- Treatment plan aur review schedule kya hoga?
- Consent, privacy aur family involvement kaise handle hoga?
- Aftercare, relapse support aur total cost kya hai?

Outpatient, intensive ya residential?

Outpatient care mein person home rehkar appointments, therapy, medication review aur support groups attend karta hai. Stable home, manageable withdrawal risk, reliable transport/support aur ability to stay safe is format ko practical banate hain. Intensive outpatient/day programmes zyada structure dete hain. Residential/inpatient care severe medical risk, unsafe environment, repeated unsuccessful outpatient attempts, complex mental-health needs, ya 24-hour monitoring requirement mein consider hota hai.

More expensive ya more restrictive automatically better nahi. NIDA treatment ko individual needs ke hisaab se tailor karne par stress karta

hai. Residential discharge ke baad bhi continuing care zaroori hai; '28 days ho gaye, ab permanent cure' expectation unsafe hai. CSBD mein bhi setting severity, safety, co-occurring conditions aur functioning par depend karti hai. Decision clinician ke saath lo, aur centre se written plan maango: goals, services, medication policy, family contact, discharge aur follow-up.

> **Good question**
> 'Is level of care ko mere case mein kyun recommend kar rahe hain? Agar main outpatient start karoon, toh kin signs par higher support chahiye hoga?'

Therapies ka practical meaning

Cognitive behavioural therapy cues, thoughts, emotions aur behaviours ke links identify karke alternative responses practise karti hai. Motivational interviewing change ke apne reasons strengthen karti hai, argument ya shame se nahi. Contingency management healthy target behaviours ko structured positive reinforcement deta hai; stimulant-use disorders mein especially strong evidence hai. Family/couples work communication, boundaries aur support improve kar sakta hai. Mutual-support groups social support de sakte hain, lekin woh medical treatment ka substitute nahi.

CSBD/PPU mein CBT-based interventions ka emerging evidence hai; ACT, mindfulness-based relapse prevention aur motivational components bhi studies mein aaye hain. Research quality aur sample diversity limited hai, isliye 'guaranteed method' claim avoid karo. Therapist se poochho: 'Mere case formulation kya hai? Kaunse skills practise hongi? Progress kaise review hoga? Agar yeh approach kaam na kare toh next option kya?' Good therapy collaborative hoti hai, humiliating nahi.

- CBT: trigger-thought-action-result mapping and practice.
- Motivational work: ambivalence ko explore karke own reasons for change.
- Contingency management: positive targets and structured reinforcement.
- Mindfulness/ACT: urges ko observe, values-based action choose karna.
- Support groups: connection and accountability, alongside—not instead of—needed medical care.

Medication se fear bhi nahi, self-prescription bhi nahi

Kuch substance-use disorders ke liye effective medications available hain. Opioid-use disorder mein buprenorphine, methadone aur naltrexone; alcohol-use disorder mein naltrexone, acamprosate aur disulfiram; tobacco cessation mein NRT, varenicline, bupropion aur cytisine jaise options different regions/approvals mein use hote hain. Correct option health, other medicines, current use, pregnancy, mental health, previous response aur patient preference par depend karta hai. Dose/timing book se decide nahi hota.

CSBD mein koi universal ‘anti-addiction pill’ nahi. SSRIs, naltrexone ya other medicines ko specialists selected cases/co-occurring conditions mein consider kar sakte hain, often off-label, aur evidence limited/developing hai. Internet se sleeping pills, anti-anxiety medication, hormones, ‘testosterone boosters’, herbal detox, ya someone else’s prescription lena risk create kar sakta hai. Agar medicine side effects de, toh abruptly stop karne ke bajay prescriber se urgent guidance lo, unless emergency advice says otherwise.

> **Medication conversation**
> ‘Is medicine ka goal kya hai? Common and serious side effects kya hain? Alcohol/other substances ke saath interaction? Missed dose/lapse par kya karna hai? Review kab hoga? Stop/taper kaise decide hoga?’

Privacy aur honest disclosure ko balance karo

Shame ki wajah se log amount kam batate, second substance chhupate, ya sexual behaviour ka important part omit karte hain. Isse withdrawal/medication risks miss ho sakte hain. Appointment se pehle confidentiality policy poochho: information kaun access karega, family ko kya bataya jayega, records kaise store honge, aur safety emergency mein disclosure kis had tak required hoga. Exact legal/policy details region/provider se vary kar sakte hain, isliye assumptions na rakho.

Agar family involve karni hai, toh consent ke saath roles define karo: appointment reminder, safe transport, emergency recognition, removing access, ya simply non-judgemental listening. Family ko password control,

invasive monitoring, physical punishment, forced public disclosure, ya medication control without clinical guidance nahi dena. Partner/family support respectful hona chahiye. Minor ho toh trusted adult aur age-appropriate qualified care important hai; unsafe adult ko confidant choose karna zaroori nahi.

- Share: facts needed for safety and treatment.
- Ask: confidentiality and its limits.
- Choose: who gets what information, where possible.
- Avoid: surveillance, threats, shame, and forced treatment details shared publicly.

Appointment-ready one-page summary

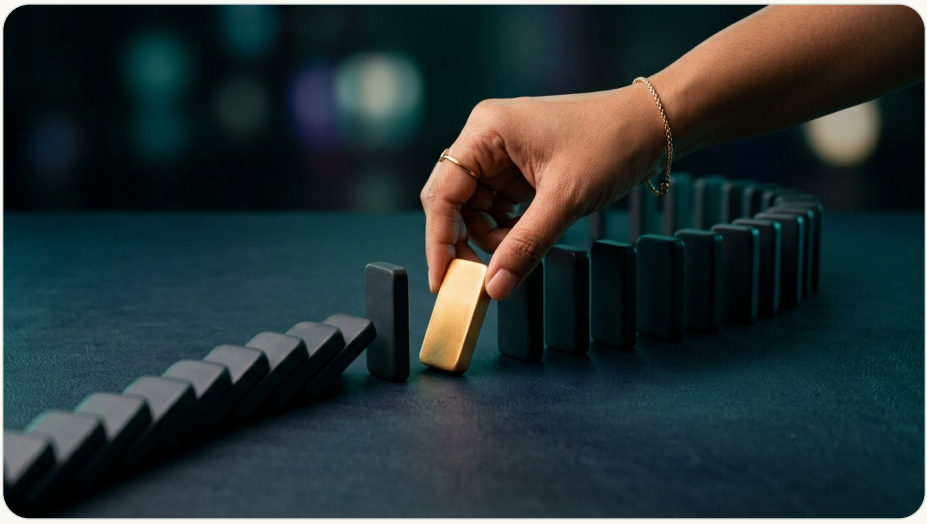
Ek single page prepare karo jise tum clinician ko dikha sako. Top par main concern; phir pattern and duration; substances/medicines; withdrawal/previous severe events; mental/physical health; concrete impacts; previous attempts; current support; and your goal. Sexual behaviour section mein non-graphic functional description enough hai. Illegality ya another person’s safety involved ho toh hide mat karo; clinician ko risk assess karna zaroori hai.

Appointment ke end par next steps repeat karke confirm karo: medicine hai toh kya, next visit kab, danger signs kya, emergency contact kaun, family ka role kya, aur lapse ho toh kya karna hai. Agar tumhe plan samajh nahi aaya, toh ‘thoda simple words mein phir se bolenge?’ kehna perfectly okay hai. Good recovery plan woh hai jo tum real life mein explain aur follow kar sako, fancy terminology nahi.

MY ONE-PAGE BRIEF

01 Main concern + desired change.
02 Pattern: type, approximate amount/frequency, duration, last episode/use.
03 Risks: withdrawal, medical, psychiatric, relationship/consent/safety.
04 Current medicines, allergies, conditions, previous treatment.
05 Next steps, warning signs, contacts and review date — clinician ke saath confirm.

PART 03



Triggers Ko Design Karna

Automatic chain ke beech mein useful pauses

Craving ka moment plan banane ka worst time ho sakta hai. Isliye hum triggers ko pehle map karenge, environment mein friction add karenge aur simple if–then responses rehearse karenge. Aim trigger-free life nahi; trigger ke saath bhi choice recover karna hai.

IN THIS PART

06 Trigger Chain Ka X-Ray
First domino ko pakdo, last action ko blame karna band karo

07 Environment Design: Access Ko Difficult, Support Ko Easy
Room, phone, money, people aur routines se gentle friction

CHAPTER 06

Trigger Chain Ka X-Ray

First domino ko pakdo, last action ko blame karna band karo

RESEARCH TRAIL R4 · R5 · R10 · R11 · R24

Chain ko backwards aur forwards dekho

Jab episode khatam hota hai, log usually last action yaad rakhte hain: ‘maine use kar liya’ ya ‘main phir wahi behaviour kar baitha.’ Chain ka useful part usse pehle hota hai. Kya notification aayi? Kya argument hua? Kya room change hua? Kya cash aaya? Kya ‘thodi der relax’ kehkar first risky step liya? Backwards trace karte hue first controllable link dhoondo. Phir forwards dekho: har link ne next link ko kaise easier banaya?

Ek chain example: difficult work call → embarrassment → alone in room → phone in hand → anonymous scrolling → permission thought → behaviour → shame → more isolation. Ab har link par alternative possible hai: call ke baad walk, room door open, phone outside, app barrier, text to support, alternate relaxation. Har link par perfect defence zaroori nahi; two or three strong pauses enough ho sakte hain. Chain mapping ka goal past ko rewrite karna nahi, future mein entry points dena hai.

CHAIN WORKSHEET

01 Final action ke turant pehle kya hua?
02 Usse pehle ka permission thought kya tha?
03 Usse pehle body/emotion state kya thi?
04 Environment/access kaunsa tha?
05 Earliest link jahan main next time different step le sakta hoon?

Body state ko ignore mat karo

HALT — hungry, angry, lonely, tired — ek simple self-check hai, diagnosis nahi. Ismein stressed, bored, unwell, physically restless aur intoxicated/withdrawal states add karna useful hai. Jab basic need unmet hai, toh brain fastest relief ko priority de sakta hai. Iska matlab har craving khana khaakar solve hogi, yeh nahi. Matlab body state ko treat karna plan ka part ho sakta hai, not irrelevant background.

Practical response small hona chahiye: regular meal, water, prescribed medication as directed, brief rest, shower, safe movement, fresh air, ya someone se ordinary conversation. Agar withdrawal possible hai, toh body symptoms ko sirf ‘urge surfing’ se manage karna unsafe ho sakta hai; medical advice priority. Similarly severe insomnia, panic, mania-like energy, ya persistent depression ko lifestyle tip se dismiss mat karo. Right response depends on what the body state actually is.

- Kya main hungry / dehydrated / exhausted hoon?
- Kya anger, loneliness, boredom ya stress spike hai?
- Kya physical pain, withdrawal ya medication effect possible hai?
- Is state ka safest direct response kya hai?
- Kya professional help add karni chahiye?

Permission thoughts: behaviour se pehle ki negotiation

Kabhi trigger direct action nahi laata; beech mein ek convincing sentence aata hai. ‘Bas ek baar.’ ‘Aaj toh deserve karta hoon.’ ‘Main kal se perfect rahunga.’ ‘Ek slip ho hi gaya, ab kya farak.’ ‘Kisi ko pata nahi chalega.’ ‘Isse stress handle hoga.’ Yeh thoughts facts ki tarah feel hote hain kyunki woh current discomfort ko immediate solution dikhate hain. Unhe argue kiye bina bhi label kiya ja sakta hai: ‘permission thought active hai.’

Counter-statement long lecture nahi hona chahiye. Short, believable line better: ‘Baad mein regret predict kar sakta hoon.’ ‘Ten-minute pause ke baad decide karunga.’ ‘Slip ka answer binge nahi.’ ‘Secret hona safety nahi.’ ‘Relief chahiye, old route compulsory nahi.’ Substance intoxication ke baad decision-making impaired ho sakti hai; tab negotiation ke bajay pre-commitment, distance aur support call zyada important hai.

Permission thought ko early catch karna strongest hai.

Write yours
Apne top three permission thoughts likho. Har ek ke saamne ek short reality-based response, one trusted action aur one person whom you can contact. Response ko ‘I must never’ se nahi, ‘I can pause’ se build karo.

If–then plans decision load kam karte hain

General intention — ‘main control karunga’ — high-pressure moment mein vague hai. If–then plan specific hota hai: ‘Agar Friday raat akela hoon aur urge aaye, toh phone ko charger par living room mein rakhkar five-minute walk aur support text karunga.’ ‘Agar friend substance offer kare, toh main “Nahi, main avoid kar raha hoon” bolunga; pressure continue ho toh place leave karunga.’ Plan ke andar action, location aur backup clear hote hain.

If–then plan ko realistic rakho. Agar gym jaana 40 minutes door hai, toh high-urge moment mein unlikely hai. Better first action two minutes mein start ho. Backup bhi rakho: walk possible nahi, toh shower; call response nahi, toh second contact; room change nahi, toh door open and sit in shared area. Substance withdrawal risk mein if–then plan medical instructions ke neeche hai, uske upar nahi. Severe symptoms = emergency protocol, not journaling.

THREE IMPLEMENTATION INTENTIONS

01 AGAR [specific trigger] ... TOH main [two-minute safe action] ...
02 Agar first action possible na ho ... TOH main [backup] ...
03 Agar control phir bhi slip ho ... TOH main [support/professional contact] ...
04 Agar emergency sign ho ... TOH main 112 / emergency help ...

Coping menu ko moment se pehle banao

Urge ke time ten ideas yaad nahi aate. Isliye ek short menu pehle se likho: physical, sensory, social, cognitive aur practical categories mein. Physical: walk, stretch, shower. Sensory: cold water on hands, breathing, music. Social: call, shared room, support message. Cognitive: permission thought label, values reminder. Practical: task with clear endpoint, making tea, organising one shelf. Menu ka aim distraction hi nahi; safe state tak time dena aur underlying need address karna hai.

Har activity ko test karo, assume mat karo. Kuch logon ke liye music trigger hai; kuch ke liye shower safe, kuch ke liye privacy increase karta hai. Substance-using friends ke saath ‘just normal hangout’ early recovery mein unsafe ho sakta hai. Sexual compulsion mein erotic content ko ‘distraction’ use karna obviously counterproductive. Menu ko non-triggering, accessible aur values-aligned rakho. Jo kaam kare usse mark karo; jo na kare usse replace karo, not yourself.

- Two actions under two minutes.
- Two actions outside the high-risk room.
- Two people/services for contact.
- One action for the underlying emotion.
- One emergency escalation if safety changes.

Composite example: Friday ka revised map

Aman — fictional composite, real patient story nahi — ko Friday late night sabse difficult lagta tha. Usne pehle sirf ‘weekend willpower weak’ likha. Chain mapping se pata chala: Friday salary/message → friends ka plan → social anxiety → alone after plan → phone in bed → permission thought → repeated behaviour → Saturday exhaustion. Ismein single ‘addiction reason’ nahi, multiple solvable links the.

Usne plan banaya: salary ke day unnecessary access reduce; friends se alcohol-free afternoon plan; Friday late night phone outside bedroom; social anxiety ke liye therapist discussion; urge aaye toh ten-minute pause and support text. Perfection immediately nahi aayi, lekin Saturday exhaustion reduce hua aur high-risk windows identifiable ho gayi. Lesson ‘Aman strong ho gaya’ nahi; lesson ‘map specific hua, toh choices visible hui.’ Tumhari chain different hogi. Example ko copy nahi, method use karo.

> **Fiction note**
> Book ke named examples fictional composites hain, sirf concepts demonstrate karne ke liye. Kisi real person ka private case ya testimonial present nahi kiya gaya.

CHAPTER 07

Environment Design: Access Ko Difficult, Support Ko Easy

Room, phone, money, people aur routines se gentle friction

RESEARCH TRAIL R4 · R10 · R11 · R13 · R17 · R18 · R24 · R26

Friction ka principle

Behaviour jitna immediately available hota hai, urge ke time utna hi fast decision lagta hai. Friction ka matlab safe obstacles: distance, time, extra step, reduced privacy for risky digital behaviour, blocked payment/access, or another person nearby. Friction punishment nahi. Tum door lock karke khud ko unsafe confinement mein nahi daal rahe; tum simply impulsive chain ko slow kar rahe ho. Best friction woh hai jo high-risk behaviour ko harder aur healthy action ko easier banata hai.

Environment change ko relevant rakho. Agar porn late-night phone se hai, toh office chair change karna enough nahi. Agar alcohol home stock se hai, toh unopened stock ka plan clinician ke overall withdrawal plan ke saath banao. Agar friend group use par pressure deta hai, toh contact boundaries central hain. Agar compulsion stress se hai, toh sirf blocker ke saath stress support bhi chahiye. Friction ek layer hai, complete treatment nahi.

> **Design test**
> Kya yeh change risky action ko genuinely slower banata hai? Kya safer action ko genuinely easier banata hai? Agar dono no, toh decoration ho sakta hai, protection nahi.

Substance environment: safe removal, not secret detox

Agar medically safe plan agreed hai, toh home/car/bag se substances aur related paraphernalia ko remove karna access kam kar sakta hai. Disposal local safety/legal guidance ke according ho; unknown substances ko others ke reach mein chhodna, flushing indiscriminately, ya hazardous handling avoid karo. Trusted person ko removal mein include karna useful ho sakta hai. Prescription medicines ko secure storage mein rakho aur exact prescribed access clinician ke saath plan karo.

Lekin heavy alcohol/sedative dependence mein stock hatana aur suddenly stop karna same decision nahi hai. Removal ke saath withdrawal plan, monitoring aur emergency contacts clear hone chahiye. Family ko secretly medicines discard karna bhi unsafe ho sakta hai. Opioids/sedatives ko abrupt hide karke ‘tough love’ dena treatment nahi. Person ko informed plan ka part banao, jab tak immediate safety requires otherwise. Clinical instructions ko priority do.

- Agreed treatment/withdrawal plan before a big change.
- Secure medicines; no shared or borrowed doses.
- Remove risky access where safe and lawful.
- Keep emergency contacts and monitoring in place.
- Do not use confiscation as humiliation or punishment.

Digital barriers: tool hai, moral police nahi

Compulsive porn/sexual scrolling mein app/site blockers, safe-search, reduced recommendations, logged-out state, no saved payment, device outside bedroom, and screen-free wind-down useful barriers ho sakte hain. Blocker ka password kisi trusted adult ke paas ho sakta hai agar tum consensually chahte ho, lekin coercive partner control ya secret surveillance healthy recovery nahi. Privacy aur dignity maintain karo. Work/school device policies follow karo; unsafe bypass tricks ki zaroorat nahi.

Barrier ko trigger chain se match karo. Agar late-night private browsing main route hai, toh daytime blocking alone enough nahi. Agar social-media suggestions entry point hain, toh unfollow/mute/reset

recommendations and reduce idle scrolling. Agar alternate device loophole hai, toh environment plan mein woh bhi include karo. Technology fail ho sakti hai; isliye blocker ke saath urge plan, therapy aur emotional coping rakho. ‘Installed = cured’ expectation avoid karo.

DIGITAL FRICTION REVIEW

01 Primary device/app/time kya hai?
02 Kaunsa two extra steps create karega?
03 Bedroom/work/sleep boundaries kya hongi?
04 Consent-based support kis role mein hoga?
05 Blocker fail/bypass ho toh safe reset kya hoga?

Money, places aur social access

Substance pattern mein cash, cards, delivery apps, particular shops, routes, bars, parties ya dealers/contact networks access ko instant banate hain. Practical steps: unnecessary cash kam, risky payment methods remove, safe route, high-risk venue avoid, substance-free plans, aur trusted person ke saath early check-ins. Legal obligations ya essential work ko ignore kiye bina alternatives banao. Agar environment violent/coercive hai, toh safety service/professional support priority; simple assertiveness enough nahi ho sakti.

Social boundaries short ho sakti hain: ‘Nahi, main avoid kar raha hoon.’ ‘Main substance-free plan mein hoon.’ ‘Main yeh conversation continue nahi karunga.’ ‘Main ab leave kar raha hoon.’ Explanation dena compulsory nahi. Agar person pressure kare, toh repeat and exit. Relationship ko preserve karna safe ho tabhi attempt karo; unsafe pressure ko relationship-saving ke naam par tolerate mat karo. Supportive people ko advance mein batao ki offer na karein aur rescue plan kya hai.

- One sentence refusal.
- One exit action.
- One alternative social plan.
- One person who knows not to offer.
- One escalation if pressure becomes unsafe.

Sleep aur bedroom ko trigger zone se nikalo

Late-night fatigue, privacy, phone in bed aur next-day no plan ek powerful combination hai. Fixed wake time, wind-down routine, device charging outside room, comfortable lighting, aur morning ke liye one visible healthy action help kar sakte hain. Agar substance sleep ke liye use ho raha hai, toh suddenly remove karna especially alcohol/sedatives mein unsafe ho sakta hai; clinician ke saath sleep plan banao. Sleep problem persistent ho toh assessment chahiye.

Masturbation/porn pattern mein bedroom ko phone-free karna helpful ho sakta hai, lekin goal body se fear karna nahi. Healthy sexuality aur private space ko shame nahi karna; target specific compulsive chain hai. Alternative wind-down — paper book, calm music, breathing, stretching, conversation — choose karo jo actually soothing ho. Agar racing thoughts, trauma symptoms, panic, ya severe insomnia hai, toh professional mental-health support add karo.

> **Night boundary**
> Mera phone ___ baje ___ jagah charge hoga. Urge aaye toh main bed se uthkar ___ location par ___ action karunga. Emergency/withdrawal symptoms ho toh device rule se pehle medical help.

Environment review after every lapse

Agar lapse hua, toh environment ko data ki tarah inspect karo. Kaunsa barrier missing tha? Kya blocker manually off hua? Kya cash available tha? Kya trusted person unavailable tha? Kya risky place already planned tha? Lapse ko prove mat banao ki environment useless hai; usse design review banao. Ek barrier fail hua ho toh doosra layer add karo, aur agar repeated bypass intentional hai toh underlying motivation/ambivalence ko therapy mein explore karo.

Environment ko too restrictive bhi mat banao. Agar plan social isolation, unsafe confinement, work loss, excessive control, ya constant monitoring create kare, toh woh sustainable nahi. Recovery ko life build karni hai, jail nahi. Best setup safe freedom ko expand karta hai: substance-free activities, healthy relationships, meaningful routine, privacy with accountability. Weekly review mein ek thing remove karo jo unnecessary friction hai, aur ek add karo jo actual high-risk link target karta hai.

FIVE-MINUTE DESIGN REVIEW

01 Kaunsa access easiest tha?
02 Kaunsa barrier actually help hua?
03 Kaunsa bypass hua aur kyun?
04 Kaunsa healthy option difficult tha?
05 Agle seven days ke liye one addition + one subtraction + one support contact.

PART 04



Nasha Chhodne Ka Practical Plan

Safe cessation, evidence-based treatment aur real-life support

Substance recovery mein determination important hai, lekin correct treatment us determination ko safe direction deta hai. Yeh part quit preparation, craving response aur substance-specific care ko cover karta hai. Kisi bhi medication ko start/stop/taper karne ka instruction yahan nahi hai — woh qualified prescriber ke saath decide hota hai.

IN THIS PART

08 Safe Quit Plan: Date Se Pehle Ka Kaam
Preparation ko delay tactic nahi, protection banao

09 Substance-Specific Guide
Same recovery value, different medical and treatment needs

10 Treatment Ko Apna Ally Banao
Medication, therapy aur support groups ko samajhkar use karo

CHAPTER 08

Safe Quit Plan: Date Se Pehle Ka Kaam

Preparation ko delay tactic nahi, protection banao

RESEARCH TRAIL R10 · R11 · R12 · R13 · R16 · R17 · R18 · R24

Goal ko clinician ke saath align karo

Kuch log complete abstinence chahte hain, kuch pehle reduction. Goal severity, substance, withdrawal risk, health, previous attempts, legal/social context aur patient preference par depend karta hai. Severe alcohol/opioid/sedative problems mein medically supervised plan especially important hai. Kisi ke ‘controlled use’ success story ko apne case par apply mat karo, aur kisi ko abstinence impose karne ke bajay qualified assessment se risks samjho. Treatment ka target meaningful health and functioning restoration hai.

Goal ko clear language mein likho: ‘Main doctor ke supervised plan ke saath alcohol-free recovery pursue karunga,’ ya ‘Main tobacco quit date set karke counselling aur recommended cessation support use karunga.’ Agar clinician reduction/taper recommend kare, toh exact schedule unke instructions mein hoga, book mein nahi. Compulsive masturbation/porn goal ko substance goal se separate rakho; dono ko same day par perfect karne ka pressure mat lo. Priority highest safety risk ko do.

> **Priority rule**
> Medical danger > immediate safety > maintaining treatment > skill practice > streak perfection. Jab goals conflict hon, toh isi order mein decide karo.

Quit date: commitment plus logistics

Quit date useful ho sakti hai jab preparation uske saath ho. Appointment, prescriptions/instructions, support person, safe environment, time off if needed, transport, food/hydration, emergency plan, high-risk events, and what to do if a lapse occurs — sab pehle se clear rakho. Date ko repeatedly postpone karna avoidance ho sakta hai, lekin unsafe detox ko ‘commitment’ kehna bhi avoidance of medical care hai. Clinician help se date choose karo.

First days ke liye schedule intentionally simple rakho. Nonessential travel, parties, high-risk meetings, major conflicts aur intense deadlines ko jitna possible ho move karo. Support person ka role written rakho: check-in times, emergency signs, safe transport, appointment reminders. Agar home unsafe ya substance-filled hai, toh clinician se higher level of care ya alternative safe setting discuss karo. Preparation ka goal comfort nahi, continuity and safety hai.

READINESS CHECKLIST

01 Medical risk assessed and plan explained.
02 Current substances/medicines shared honestly.
03 Support person and check-in times fixed.
04 High-risk access reduced where safe.
05 Emergency numbers and nearest facility known.
06 Lapse/missed dose instructions confirmed with prescriber.

First days: basic routine, low demand

Early recovery mein body aur mood unpredictable ho sakte hain. Sleep disturbance, irritability, anxiety, low mood, cravings, appetite changes, restlessness ya concentration issues substance ke according vary kar sakte hain. Har symptom ko same withdrawal label dena bhi accurate nahi; clinician ko severe/new/persistent symptoms batao. Routine simple rakho: prescribed plan, regular meals as tolerated, hydration, safe rest, brief check-ins, low-pressure activities, and avoiding driving or risky tasks when impaired.

‘Busy raho’ ko extreme mat banao. Overwork, punishing exercise, starvation, caffeine overload, ya nonstop social contact exhaustion badha sakta hai. Gentle structure better: morning check-in, one necessary task,

one supportive conversation, one calming activity, evening review. Agar person unable to keep fluids, severely confused, hallucinating, having seizure, breathing abnormally, or suicidal, toh routine nahi — emergency care. Self-care never replaces red-flag response.

- Follow prescribed/monitoring plan exactly.
- Keep demands realistic and avoid intoxicated/impaired driving.
- Use regular simple meals/hydration as tolerated and medically advised.
- Check in with support person even when you feel ‘okay’.
- Escalate new, severe or worsening symptoms promptly.

Craving ke time five-step response

Step one: **Ruko** — current action ko pause karo, substance/phone/access se distance create karo if safe. Step two: **Ground** — feet on floor, slow comfortable breathing, five things you see. Step three: **Label** — ‘craving hai, order nahi’ and name the trigger. Step four: **Act** — preselected two-minute action: leave place, call, shower, walk, eat if appropriate. Step five: **Contact** — support person/clinician/quitline, especially if craving is intensifying or plan is failing. Yeh sequence emergency symptoms ke liye nahi; red flags mean emergency help.

Craving ko zero banana goal nahi; action ke beech gap create karna goal hai. Agar first step fail ho, toh next available step par jump karo. ‘Pause nahi hua, ab kya point?’ permission thought hai. Substance already consumed ho toh additional use avoid, safe place/person, no driving, and assess medical risk. After the wave, two lines record karo: kya trigger tha, kaunsa step help hua. Repeated severe cravings treatment review ka signal ho sakte hain, character flaw nahi.

POCKET CARD

01 RUKO — access/situation se pause.
02 GROUND — breathing + senses.
03 LABEL — trigger and permission thought.
04 ACT — two-minute safe alternative.
05 CONTACT — person/service; emergency signs par 112.

Social pressure ka rehearsal

Offer ke moment par long explanation dena often negotiation open karta hai. Short line rehearse karo: ‘Nahi, main avoid kar raha hoon.’ ‘Main recovery plan follow kar raha hoon.’ ‘Mere liye substance-free evening better hai.’ ‘Main drive kar raha hoon’ only if true — false excuses avoid karo. Person respect kare toh topic change; pressure ho toh repeat and exit. Agar safety threat hai, toh confrontation ke bajay safe exit/help choose karo.

Supportive friends ko exact instruction do: ‘Mujhe offer mat karna,’ ‘Agar main isolate hoon toh call karna,’ ‘Mujhe ride/home safe chahiye,’ ‘Meri medical details others ko mat batana.’ Social recovery mein substance-free activities add karo — chai, walk, sport appropriate to health, movies, classes, volunteering — taaki calendar empty na rahe. Abstinence social death nahi honi chahiye; new routines ko time do, aur people choose karo jo boundaries respect karte hain.

- One-line no.
- One topic change.
- One exit line.
- One safe person to contact.
- One alternative plan you actually enjoy or value.

Daily review, weekly treatment review

Daily review two minutes: craving high kahan thi, kaunsa skill use hua, kya medication/appointment instructions follow hue, any safety issue, tomorrow ka one adjustment. Obsessive checking avoid karo. Weekly review clinician/support person ke saath: pattern improve ho raha hai? Side effects? Sleep/mood? Access? Work/relationships? New risk? Plan adjust karo. Treatment ko continue karna important hai; early improvement par suddenly stop karna risk create kar sakta hai.

Progress ko multi-dimensional dekho: reduced use/abstinence as chosen, fewer high-risk episodes, better health, improved attendance, repaired sleep/work, less secrecy, safer relationships. Return to use ho toh review urgent bana do; hiding se plan irrelevant ho jaata hai. NIDA treatment plans ko ongoing review aur modify karne par stress karta hai. Tumhara plan static contract nahi, living document hai.

WEEKLY SEVEN QUESTIONS

01 What improved?
02 What was hardest?
03 Any use/lapse or near-miss?
04 Any side effects/withdrawal/medical concerns?
05 Which trigger was underestimated?
06 Which support actually helped?
07 What changes before next week?

CHAPTER 09

Substance-Specific Guide

Same recovery value, different medical and treatment needs

RESEARCH TRAIL R10 · R11 · R13 · R14 · R15 · R16 · R17 · R18 · R19 · R20 · R22 · R24

Tobacco aur nicotine

Cigarettes, bidis, smokeless tobacco, hookah, vape/nicotine products ke patterns different ho sakte hain, lekin evidence-based cessation support available hai. WHO behavioural counselling, digital support as adjunct, and medications such as varenicline, NRT, bupropion and cytisine ko recommend karta hai for appropriate adults; combination of behavioural and pharmacological support often improves outcomes. Product availability/approval vary by country, isliye qualified clinician se option choose karo. Smokeless tobacco ke liye bhi intensive behavioural support useful hai.

India’s National Tobacco Quitline **1800-11-2356** counselling aur quitting strategies ke liye verified government resource hai. Plan mein morning/coffee/tea/after-meal/stress cues, social offers, stock removal, and alternative hand/mouth routines include karo — without replacing with another harmful substance. NRT ko ‘same addiction’ samajhkar reject na karo; correctly used treatment uncontrolled smoking ke same pattern nahi. Prescriber se pregnancy, heart conditions, mental health, other medicines, and side effects discuss karo.

- Set a supported quit plan and identify top cues.
- Use behavioural + medication options when clinically appropriate.
- Call National Tobacco Quitline 1800-11-2356.
- Do not switch to another tobacco/nicotine product assuming it is harmless.
- Review lapses early rather than abandoning the plan.

Alcohol

Alcohol-use disorder treatable hai, aur behavioural healthcare, medications aur mutual-support options different combinations mein help kar sakte hain. NIAAA naltrexone, acamprosate aur disulfiram ko approved treatment options batata hai in relevant settings; clinician suitability decide karta hai. Heavy prolonged use ke baad abrupt stopping potentially life-threatening withdrawal produce kar sakti hai. Isliye pehla question ‘kitni willpower?’ nahi, ‘withdrawal risk kya hai?’ hona chahiye.

Seizures, hallucinations, severe confusion, delirium, fainting, severe agitation, or inability to stay safe = emergency. Treatment mein outpatient/inpatient withdrawal management, relapse-prevention therapy, medication review, sleep/mental-health care and ongoing support include ho sakta hai. Detox alone long-term plan nahi. Driving, swimming, operating machinery, caregiving alone, or mixing alcohol with opioids/sedatives high-risk hai. Social events ke liye alcohol-free alternatives and exit plan rehearse karo.

> **Alcohol safety**
> Heavy use + withdrawal history + other sedatives + medical illness = urgent professional assessment before abrupt change. Never use another sedative or someone else’s prescription to self-manage withdrawal.

Cannabis

Cannabis-use disorder ke liye behavioural therapies — CBT, motivational enhancement, contingency management and relapse-prevention work — central hain. NIDA currently no FDA-approved medication specifically for cannabis-use disorder batata hai. Kuch log stopping ke baad irritability, sleep change, anxiety, appetite change or cravings notice karte hain; support and routine help kar sakte hain, but severe/persistent symptoms assess karwao. Product potency and mixed tobacco use bhi pattern ko affect kar sakte hain.

Using for sleep, anxiety, low mood or social discomfort ho toh underlying issue ko treatment plan mein include karo, otherwise removal ke baad same need return karegi. Severe anxiety, paranoia, hallucinations, psychosis, chest symptoms, repeated vomiting, or inability to stay safe ke liye urgent medical care chahiye. ‘Natural hai, harmless hai’ assumption

avoid karo; legal status bhi location ke hisaab se consider karo. Driving/unsafe tasks while impaired avoid karo.

- Identify function: sleep, anxiety, boredom, social bonding, or other relief.
- Use behavioural treatment and alternative coping, not unproven detox products.
- Include tobacco/mixed-substance use in assessment.
- Seek urgent help for psychosis, severe physical symptoms or safety risk.

Opioids

Opioid-use disorder ke liye medication first-line ho sakti hai, usually behavioural therapy/counselling and social support ke saath. Buprenorphine, methadone and naltrexone effective options hain; correct choice and initiation timing qualified clinician decide karta hai. NIDA medication treatment retention, reduced use and reduced overdose/infectious risk se link karta hai. Detox alone often insufficient, and stopping maintenance early without clinical plan can increase risk.

Reduced tolerance after abstinence/reduced use relapse overdose risk badha sakti hai. Unresponsiveness, very slow/irregular breathing, blue/pale lips, or inability to wake = call emergency immediately. Alcohol, benzodiazepines or other depressants ke saath mixing risk ko increase kar sakta hai. Naloxone opioid overdose reversal medication hai, but availability/training/local protocols vary; emergency call ko replace nahi karta. Never borrow methadone/buprenorphine or self-adjust doses.

> **Key truth**
> Prescribed opioid-agonist treatment is not simply ‘replacing one addiction with another.’ Under supervision it can stabilise withdrawal/craving and support recovery. Unprescribed use and medication changes are different risks.

Sedatives, sleeping pills aur anti-anxiety medicines

Benzodiazepines and related CNS depressants can create physical dependence, even when originally prescribed. NIDA in substances ko apne aap abruptly stop na karne ki warning deta hai because withdrawal can be severe and potentially life-threatening. Medically supervised detox/taper may be needed; exact schedule individualised hota hai. Book, friend, forum ya previous patient ka taper copy mat karo. Other opioids/alcohol ke saath combined use breathing/overdose risk badhata hai.

Agar dose repeatedly increase ho rahi hai, early refill chahiye, multiple prescribers involved, medicine for intoxication use ho rahi hai, or sedation causing falls/accidents, toh honest review urgent hai. Doctor ko current actual amount and source batao. Sleep/anxiety ke alternative evidence-based treatments available ho sakte hain, but switching/stopping suddenly is not the answer. Severe confusion, seizure, hallucinations, fainting, breathing issues or suicidal thinking = emergency.

- No self-made taper, substitution or double dose.
- Share every prescriber, pharmacy and non-prescribed source as relevant.
- Avoid alcohol/opioid mixing and impaired driving.
- Ask for supervised plan plus underlying anxiety/sleep treatment.

Stimulants

Cocaine, methamphetamine and misused prescription stimulants ke liye behavioural therapies central hain; contingency management especially evidence-supported hai. NIDA currently no FDA-approved medication for stimulant-use disorder batata hai. Treatment mein cue management, structured routine, healthy rewards, sleep/mental-health support and relapse prevention important hain. Prescription stimulant medically indicated ho toh bhi misuse/dose change prescriber ke saath address karo, not secretly.

Intense agitation, paranoia, hallucinations, chest pain, severe headache, overheating, seizure, or violent/unsafe behaviour emergency signs hain. ‘Come down’ handle karne ke liye alcohol, sedatives or other drugs use karna additional risk create kar sakta hai. SAMHSA Matrix approach structured outpatient sessions, relapse prevention, social support and

family education use karta hai; aise structured programmes ask about. Recovery mein sleep and mood may need time and professional review.

> **Avoid the ‘balancing’ myth**
> Ek substance ke effects ko doosre se manage karna self-treatment nahi; polysubstance risk badha sakta hai. Tell the clinician everything used.

Inhalants aur unknown substances

Inhalants and unknown/unregulated products unpredictable medical risk create kar sakte hain, including sudden severe complications. Product name known na ho toh bhi clinician ko description, packaging if safely available, approximate time/amount and symptoms batao. Substance ko smell/taste/test karke identify karne ki koshish mat karo. Collapse, breathing difficulty, chest symptoms, seizure, severe confusion or unresponsiveness = emergency.

Behavioural treatment and comprehensive assessment important hain because no single approved medication covers every inhalant. Unknown pills/powders bought online or shared by others ko ‘same as prescription’ assume mat karo. Counterfeit products may contain unexpected substances. Safe disposal local authority/pharmacist guidance se karo; keep away from children and others. If illegal supply/network involvement creates danger, prioritise personal safety and qualified/legal support.

- Do not test unknown substances on yourself.
- Emergency symptoms require immediate care.
- Give clinicians honest product/time/symptom information.
- Use professional assessment, not internet identification or home detox.

Polysubstance use: full picture dena zaroori

Alcohol + opioids, opioids + sedatives, alcohol + sedatives, stimulants + depressants, or nicotine/cannabis alongside other use can change withdrawal, overdose and medication decisions. Kabhi primary substance obvious nahi hota. Treatment plan ko full sequence samajhna hai: kya use hota hai, kis order mein, kis reason se, aur kis combination mein. ‘Main problem sirf alcohol hai’ kehkar sleeping pills chhupana unsafe ho sakta hai.

Integrated assessment mein mental health, pain, sleep, trauma, relationships and sexual behaviour bhi relevant ho sakte hain. Substance use ke time consent impaired ho sakta hai; sexual risk/non-consensual situations ko bhi clinician ke saath safety planning mein include karo. Har substance ko alag doctor ko partial information ke saath treat karwana interactions miss kar sakta hai; prescriber list share karo. One coordinated plan is safer than disconnected instructions.

MY FULL-USE TIMELINE

01 All substances/medicines used, including alcohol and sleep aids.
02 Approximate amount/frequency and last-use times.
03 Combinations and reasons for each.
04 Previous withdrawal/overdose/mental-health events.
05 All prescribers/services involved and next coordinated appointment.

CHAPTER 10

Treatment Ko Apna Ally Banao

Medication, therapy aur support groups ko samajhkar use karo

RESEARCH TRAIL R10 · R11 · R12 · R14 · R16 · R17 · R18 · R24 · R25

Medication = crutch nahi, treatment tool

Kuch log medication ko weakness ya ‘real recovery ke against’ samajhte hain. Evidence-based prescribing ka aim intoxication chase karna nahi, withdrawal/craving manage karna, functioning improve karna, harm reduce karna and therapy mein engage hona possible banana hai. Opioid-use disorder mein medication especially important ho sakti hai; alcohol/tobacco mein bhi options exist karte hain. Prescribed, monitored treatment aur uncontrolled use ko same label dena scientifically aur practically misleading hai.

Medication ka benefit ho toh bhi side effects, interactions, adherence and review matter karte hain. Dose adjust, switch, stop ya restart sirf prescriber ke saath. Agar stigma family se aa raha hai, toh clinician se educational material/request discussion lo. Recovery ko purity test mat banao. Agar treatment se person safer, more stable, more present and healthier ho raha hai, toh woh meaningful outcome hai.

> **Dignity in treatment**
> Tumhe treatment ke liye prove nahi karna ki tum ‘sach mein change’ chahte ho. Treatment follow karna hi change ka behaviour hai.

Therapy ko passive appointment mat banao

Therapy mein sirf weekly venting se zyada value tab milti hai jab between-session practice hoti hai. Trigger map laao, actual situations discuss karo, refusal rehearse karo, thought record complete karo, and what did not work honestly batao. Agar therapist ka approach samajh nahi aata ya fit nahi hota, toh respectful feedback do. Fit na hone par alternative qualified provider possible ho sakta hai; abandoning all help is not the only option.

Therapeutic relationship safe honi chahiye: no sexual advances, humiliation, threats, financial exploitation, miracle guarantees, or pressure to stop prescribed medicines without medical coordination. Confidentiality boundaries clear honi chahiye. CSBD/sexual concerns mein especially important hai ki therapist consensual adult sexuality ko shame na kare and actual impaired control/harm par focus kare. You are allowed to ask about training and experience.

- Bring one real situation, not only general talk.
- Practise one skill between sessions.
- Report side effects, lapses and near-misses.
- Ask how progress is measured and treatment adjusted.
- Expect respect, consent and professional boundaries.

Peer support: useful complement, personal choice

AA/NA-style groups, secular recovery groups, professionally facilitated groups, and trusted peer communities connection de sakte hain. Shared experience isolation kam karta hai and practical coping ideas mil sakte hain. Lekin group medical care, detox, psychotherapy or medication management replace nahi karta. Har group culture fit nahi hota; kuch spiritual, kuch secular, kuch structured. Tum different options try kar sakte ho without declaring yourself hopeless.

CSBD/sexual behaviour groups mein extra caution: shame-based labels, explicit sharing that triggers others, moral policing, and unqualified advice can be harmful. Group should respect consent, privacy and evidence-based professional care. Online anonymity can help disclosure

but also privacy risk; identifying details, images, contacts, and illegal/explicit content share mat karo. If a group pressures you to stop medication or cut off family/professionals, treat that as a red flag.

- Does the group increase honesty and support?
- Are boundaries, privacy and dignity respected?
- Does it support—not undermine—professional treatment?
- Can you leave without threats or humiliation?
- Are practical next steps safer after attending?

Side effects aur lapse par communication

Side effect ke time two mistakes common hain: secretly stop medication, or silently endure severe symptoms. Better: record symptom, start time, severity, other substances/medicines and impact; contact prescriber. Severe allergic reaction, breathing issue, chest pain, fainting, seizure, suicidal thoughts, or other emergency signs = urgent care. Routine but bothersome effects bhi review deserve karte hain; alternatives/adjustments clinician decide karega.

Lapse/missed use/dose information ko appointment tak hide karna plan ko unsafe bana sakta hai. Call the service and ask what to do. Naltrexone with opioids, disulfiram with alcohol, sedative interactions, or agonist-treatment timing jaise issues generic advice se solve nahi hote. Do not double dose or improvise. After stabilisation, review trigger and support, then continue/revise treatment rather than declaring the whole approach failed.

> **Keep the instruction simple**
> ‘Kya hua + kab + current symptoms + kya medicines/substances involved + ab kya karna hai?’ Yahi five facts support person/clinician ko quickly do.

Progress ko wider outcomes se dekho

Substance-free days meaningful ho sakte hain, lekin recovery unse bigger hai. Reduced overdose risk, treatment adherence, improved liver/physical health where applicable, safer relationships, less secrecy, work/study stability, emotional regulation and quality of life bhi outcomes hain. Agar use reduce hua but risks remain, toh plan still needs work. Agar lapse hua but person immediately sought help and prevented escalation, that response is a meaningful safety improvement, though not a reason to ignore the lapse.

Clinical monitoring may include appointments, self-report, physical checks, medication review, and where appropriate toxicology. Monitoring ka goal punishment nahi, accurate care. Family ko progress scorecard banane ke bajay supportive role do. Celebrate actions: attended session, refused offer, called early, told the truth, returned after lapse. These behaviours make long-term recovery more likely.

MY RECOVERY OUTCOMES

01 Health and safety
02 Substance/behaviour control
03 Treatment participation
04 Sleep/work/study
05 Relationships and consent
06 Mood, meaning and quality of life

Continuing care: early improvement ke baad bhi

Detox, first month, residential stay, ya initial abstinence achievement ke baad support suddenly zero karna common risk hai. Continuing care appointments, booster sessions, peer support, medication reviews, lifestyle routine and updated relapse-prevention plan include kar sakta hai. Frequency needs ke hisaab se change hogi; support reduce karna bhi planned process ho, disappearance nahi. New job, relationship, travel, illness or grief aaye toh plan revisit karo.

Recovery ka end goal permanent vigilance nahi, sustainable life hai. Over time some skills automatic ho sakte hain, but high-risk changes ke time support temporarily increase karna wise hai. Annual/periodic health

review, mental-health care and honest self-check useful rehte hain. You do not need to wait for crisis to re-contact a clinician. Early tune-up is often easier than full restart.

Discharge question
‘Main stable feel kar raha hoon’ ke baad bhi poochho: ‘Next three months ka contact plan kya hai? Kin early warning signs par main sooner call karoon? Which medicines/skills continue?’

PART 05



Masturbation / Porn Compulsion Se Choice Wapas

No shame, no pseudoscience — control, values aur real-life repair

Yeh part un logon ke liye hai jinhe masturbation, porn use ya related sexual behaviour mein repeated loss of control aur real-life impact feel hota hai. Agar issue sirf moral guilt hai, toh bhi yahan ka shame-free framework useful hoga. Goal body ya desire ko dushman banana nahi; informed choice, consent, health aur functioning restore karna hai.

IN THIS PART

11 Compulsion Ka Personal Recovery Plan
Behaviour ko precisely define karo, phir chain ke saath kaam karo

12 Healthy Sexuality, Consent Aur Relationships
Recovery ka goal fear nahi, respectful choice hai

CHAPTER 11

Compulsion Ka Personal Recovery Plan

Behaviour ko precisely define karo, phir chain ke saath kaam karo

RESEARCH TRAIL R1 · R2 · R3 · R4 · R5 · R6 · R7 · R8 · R26

Target ko specific rakho

'Mujhe sab kuch band karna hai' vague ho sakta hai. Kya target porn viewing hai, compulsive masturbation, repeated seeking, risky chats, paid content, work-time behaviour, ya combination? Kya privacy, consent, injury, relationship agreement, sleep, work, money ya mental distress affected hai? Exact target choose karne se intervention clear hota hai. Example: 'Main late-night porn scrolling aur work-time sexual behaviour ko stop karna chahta hoon; masturbation par long-term goal therapist ke saath values-based decide karunga.'

Target ko shame words se nahi, observable terms mein likho. 'Main disgusting hoon' target nahi. 'Main weekdays 1am ke baad phone se explicit content seek karta hoon aur sleep lose karta hoon' observable hai. Isse phone boundary, bedtime routine, stress management, and review possible hain. Agar behaviour illegal/non-consensual direction mein ja raha hai, toh safety and professional/legal help immediate priority hai; self-help exercises alone sufficient nahi.

BEHAVIOUR DEFINITION

01 Target behaviour (non-graphic): ___
02 Main high-risk context: ___
03 Concrete impact: ___
04 My chosen boundary and why: ___
05 Safety/professional concern, if any: ___

CBT cycle ko real example se decode karo

CBT-style map: situation → thought → emotion/body → urge → action → short-term result → long-term result. Example: deadline criticism → 'main failure hoon' → shame/tension → escape urge → private scrolling → brief numbness → lost time and more shame. Ab intervention har point par possible hai: criticism ko specific feedback mein reframe, shame ko label, body ko ground, urge ko delay, phone ko barrier, support ko contact, and long-term result ko visible reminder.

Goal thought ko positive lie se replace karna nahi. 'Main best hoon' believable nahi hoga. Better: 'Criticism hurt hui; iska matlab whole identity failure nahi. Main next task define kar sakta hoon.' Urge ko bhi fact vs command mein separate karo: 'Mera body/attention sexual escape chahta hai; main ten minutes observe kar sakta hoon.' Practice low-intensity moments mein karo taaki high-intensity moment mein sequence familiar ho.

- Situation: what happened?
- Thought: what did I tell myself?
- Feeling/body: what changed?
- Action: what did I do next?
- Result: immediate relief and later cost?
- New response: one believable thought + one safe action.

Urge ko fight nahi, time-bound experience samjho

Urge ko suppress karne ki koshish kabhi attention ko aur fixate kar sakti hai. Alternative: notice and allow without acting. 'Mere chest/stomach/jaw mein tension hai; attention sexual direction mein ja rahi hai; urge 8/10 lag rahi hai; main isse observe kar sakta hoon.' Breathing ko forcefully slow nahi karna if uncomfortable; comfortable exhale and grounding enough. Ten-minute timer optional, lekin clock-watching compulsive ban jaaye toh use activity-based transition: make tea, step outside, wash face, call someone.

Urge surfing evidence-informed skill hai, but guaranteed immediate drop nahi. Kabhi urge remain karegi while you choose another action. That is

still success. Movement, cold water on hands, changing room, sitting in shared space, and a non-triggering task can create distance. If fantasy continues, gently redirect to sensory present rather than debating or shaming yourself. Repeated uncontrollable urges despite skills = therapist review, not self-condemnation.

Two-minute script
'Yeh urge hai, order nahi. Mujhe isse enjoy ya hate karna zaroori nahi. Main next two minutes mein safe action choose karunga. Baad mein review karunga; abhi decision automatic nahi hoga.'

Delay ke saath decision quality improve karo

Simple 'ten-minute delay' tab kaam karta hai jab us delay mein environment change aur action ho. Same room, same screen, same content ke saath timer dekhna often negotiation ko extend karta hai. Delay = stand up + device away + different location + support/alternative. If urge returns, repeat or escalate. Delay ka goal forbidden fruit ko glorify karna nahi, automatic sequence ko interrupt karna hai.

Aapka chosen boundary abstinence ho ya controlled behaviour, delay informed choice ko space deta hai. However, if controlled masturbation repeatedly becomes multi-hour loss of control, then temporarily stronger boundaries with therapist support may be useful. If complete abstinence creates obsessive panic, counting and shame, then goal itself needs review. There is no one-size-fits-all rule. Measure whether the plan increases agency and functioning without creating new harm.

DELAY DESIGN

01 My delay length (flexible, not a magic number): ___
02 Device/access change: ___
03 Location change: ___
04 Safe action: ___
05 If it fails: whom do I contact / what professional step follows?

Attention aur fantasy ko handle karna

Sexual thoughts spontaneous ho sakte hain; unka hona behaviour ke barabar nahi. Thought suppression — 'yeh sochna hi nahi chahiye' — monitoring aur guilt badha sakta hai. Instead label: 'sexual thought', then return to current task. No need to analyse every thought's meaning. If thoughts are intrusive, ego-dystonic, repetitive with compulsive checking, or linked to OCD/trauma, then specialised assessment important hai; treating them simply as desire can worsen the issue.

Mindfulness yahan explicit imagination nahi, present-moment attention hai: feet, sounds, breath, texture, ordinary task. Grounding ko ritual mat banao jise repeatedly perform karna compulsory ho jaaye. Flexible skills better. Agar particular memories/trauma trigger ho rahe hain, toh trauma-informed therapist support lo. You are not required to disclose graphic detail to friends or support groups; a qualified clinician can work with non-graphic description and safety needs.

- Thought ≠ action.
- Notice, label, gently redirect.
- Avoid compulsive checking or punishment.
- Intrusive/trauma-linked/OCD-like patterns deserve specialised assessment.
- Consent and legality remain non-negotiable in any real-world behaviour.

Underlying need ko real response do

Agar pattern loneliness, rejection, boredom, performance anxiety, relationship conflict, depression, trauma, or work stress se closely linked hai, toh blockers alone incomplete honge. Need ko name karo: connection, rest, comfort, achievement, autonomy, reassurance, or escape. Phir us need ke two non-compulsive responses choose karo. Loneliness = safe conversation/community; stress = task breakdown/therapy; boredom = engaging activity; exhaustion = sleep routine; relationship conflict = structured communication/counselling.

Alternative immediately same intensity of relief na de, tab bhi worthwhile hai. You are building a broader coping system, not finding a new instant anaesthetic. If depression, anxiety, bipolar symptoms, OCD, ADHD, trauma, or substance use co-exist, integrated treatment can matter.

Antons et al. review and related clinical work CSBD ko other disorders ke context mein assess karne par stress karte hain. Treating the whole person is not a bonus; it is often central.

NEED-TO-RESPONSE TABLE

01 Need I usually try to meet: ___
02 Old short-term route and later cost: ___
03 Two safer responses I will actually try: ___
04 Professional/relationship support needed: ___
05 How I will know the need is being better met: ___

Professional plan: kya expect karna realistic hai

CSBD/PPU treatment research developing hai. Antons et al. ke preregistered review mein 24 studies, only four RCTs; overall quality low and samples often male. Recent reviews CBT-based interventions ko promising batate hain, but certainty and long-term evidence limited hain. Isliye therapist ke saath realistic goals, progress review and flexibility important hain. Beware of programmes claiming guaranteed seven-day cure or universal hormonal reset.

Treatment may include psychoeducation, motivational work, CBT, ACT/mindfulness skills, relapse prevention, co-occurring disorder treatment, relationship work, and sometimes specialist-prescribed medication. Medication often off-label in CSBD context; evidence not same as substance-specific approved treatments. Ask about risks, alternatives and review. You can seek second opinion from a qualified professional if plan feels unsafe or shaming. Improvement may be gradual; each reduction in impairment and increase in choice matters.

Evidence check
Promising ≠ proven cure. Limited evidence ≠ no hope. It means choose qualified care, track real outcomes, avoid grand claims, and revise when needed.

CHAPTER 12

Healthy Sexuality, Consent Aur Relationships

Recovery ka goal fear nahi, respectful choice hai

RESEARCH TRAIL R1 · R2 · R3 · R4 · R6 · R8 · R9 · R26

Consent ko centre mein rakho

Consent informed, freely given, specific, reversible and capacity-compatible hona chahiye. Intoxication, fear, pressure, manipulation, power imbalance, unconsciousness or inability to understand/consent ke context mein sexual activity unsafe/non-consensual ho sakti hai. Silence, past relationship, marriage, previous yes, or arousal ko automatic consent na samjho. Kisi bhi point par no/stop respect karna mandatory hai. Yeh rule recovery ke kisi bhi phase mein change nahi hota.

Agar urges kisi non-consensual, illegal, exploitative ya minor-involving direction mein ja rahi hain, toh immediately distance, barriers and urgent qualified professional/legal help lo. Potential victim ki safety priority hai. Is book mein aise behaviour ko manage karne ke techniques nahi diye gaye; specialist risk assessment chahiye. If someone is in immediate danger, call emergency services. Healthy recovery never treats another person as a coping tool.

> **Non-negotiable**
> No technique, urge, relationship status or substance use excuses non-consensual behaviour. Safety, legality and the other person’s autonomy come first.

Partner se conversation: facts, needs, boundaries

Agar relationship affected hai, toh conversation calm, non-intoxicated moment mein karo. Structure: concrete facts without unnecessary graphic detail, impact, responsibility, current plan, and what support you are requesting. Example: ‘Late-night porn use se sleep aur hamara time affected hua hai. Main therapy aur device boundaries start kar raha hoon. Main tumse monitoring nahi, but agreed check-in chahta hoon.’ Partner ko hurt, angry, cautious ya unwilling to participate feel hone ka right hai.

Disclosure ka scope safety, health, agreements and professional guidance ke hisaab se decide hota hai. STI risk, shared finances, consent violations, or safety issues involved hon toh relevant disclosure and professional/legal support important hai. Graphic details trauma create kar sakte hain and recovery ke liye necessary nahi. Couples therapist structured conversation mein help kar sakta hai. Partner ka role detective, parent or 24/7 blocker banana automatic nahi.

- What happened at a functional, non-graphic level?
- What impact do I take responsibility for?
- What concrete plan is in place?
- What am I asking for, and what can partner decline?
- What boundaries/safety/health concerns need separate attention?

Agreements ko explicit banao

Relationships mein masturbation/porn ke views vary karte hain. Koi behaviour apne aap mein universally ‘cheating’ ya ‘not cheating’ label se settle nahi hota; agreements, consent, honesty and impact matter karte hain. Dono partners apne boundaries express kar sakte hain, but boundaries coercion nahi ban sakti. Agreement specific ho: kya acceptable, kya private, kya shared expectation, kya digital/financial limit, and what happens if agreement breaks.

Agreement mein punishment, forced access to all passwords, public exposure, sexual obligation, or threats include nahi hone chahiye. ‘If you loved me you would never have desire’ unrealistic. Better: ‘We agree no work-time porn and no paid content; if lapse happens, we discuss impact

and review support.’ If partners’ values fundamentally differ, counselling can clarify compatibility. Separation may be a valid choice; no one owes staying in a harmful dynamic.

AGREEMENT CHECKLIST

01 Specific behaviour and context.
02 Reason and value behind the boundary.
03 Privacy vs accountability distinction.
04 Health, consent and financial safeguards.
05 Repair/review process after disagreement or lapse.
06 Each person’s right to say no and seek support.

Partner ko accountability police mat banao

Accountability helpful ho sakti hai: agreed weekly check-in, therapy appointment encouragement, shared device-free time, or support during high-risk stress. Lekin continuous checking, interrogation, location tracking, forced body checks, humiliation, or using sex as reward/punishment unhealthy ho sakta hai. Person with compulsive behaviour ko responsibility apni leni hogi; partner cannot carry entire control system. Partner ki own mental health and freedom bhi matter karti hain.

If monitoring is used, it should be consensual, time-limited, reviewable, minimally invasive, and paired with skill-building. Either person can request revision. If there is fear, violence, coercive control or retaliation, monitoring plan unsafe ho sakta hai; specialist domestic-safety support priority. Recovery must not become another arena for abuse. Professional couples work should screen for safety, not push joint sessions blindly.

Healthy accountability

‘Main tumhe weekly update dunga aur plan follow karunga’ is different from ‘Tum meri har movement check karo aur mujhe control karo.’ Responsibility stays with the person; support remains consensual.

Body trust aur non-sexual connection rebuild karo

Compulsion ke baad body se disconnection, performance anxiety, touch avoidance, or sex ko test banane ka pressure ho sakta hai. Recovery mein sexual performance ko immediate target banana counterproductive ho sakta hai. Start with non-sexual connection: conversation, shared meals, walk, hobbies, appreciation, and affection only where mutually comfortable. No expectation that affection must lead to sex. This reduces pressure and makes consent clearer.

If sexual pain, erectile/lubrication/orgasm concerns, genital symptoms, trauma responses, or marked distress exist, qualified medical/sexual-health professional help lo. Do not assume masturbation is the cause of every sexual difficulty; causes can be medical, psychological, relational, medication-related or mixed. Online supplements and unproven ‘nerve repair’ products avoid karo. A clinician can assess without shame.

- Remove performance tests and deadlines.
- Prioritise non-sexual trust and ordinary companionship.
- Keep touch optional and consent-based.
- Seek medical/sexual-health assessment for persistent symptoms.
- Avoid miracle supplements and blame-based explanations.

Relationship harm ke baad repair ka matlab

Repair sirf sorry nahi; acknowledgement, responsibility, safety, restitution where relevant, consistent behaviour and respect for the other person’s pace hai. ‘Maine relapse kar liya, tumhe forgive karna chahiye’ pressure hai. Better: ‘Maine agreement toda; impact tum par bhi hai. Maine yeh steps liye hain; tum jo boundary decide karte ho usse respect karunga, and professional support continue karunga.’ Forgiveness ko demand nahi kiya ja sakta.

Sometimes repair means accepting distance or relationship ending. Sometimes it means structured counselling and gradual trust. Repeated promises without action trust ko further damage karte hain. Let observable actions speak: appointments, transparent agreed finances, safer digital habits, honest lapse plan, respectful communication. But

transparency should not become total loss of privacy. Repair is a process, not a single emotional conversation.

REPAIR STATEMENT

01 I acknowledge the concrete impact: ___
02 I take responsibility without blaming stress/partner/urge: ___
03 Actions already taken: ___
04 Next professional/safety step: ___
05 I respect your boundary and your right to time/support: ___

Your worth is not your behaviour history

Compulsive behaviour ke baad log khud ko permanently undesirable, unsafe or unlovable label kar sakte hain. Responsibility ke saath dignity maintain karna possible hai. If you have harmed someone, their boundaries and safety matter; simultaneously, you still need qualified help to change. Self-hatred is not a safeguarding strategy. Stable accountability, treatment, empathy and consistent action are more useful.

A healthy sexual life, if you want one, is based on consent, communication, mutual respect, health and choice — not frequency records or performance myths. If you choose abstinence, that can also be a valid values-based life. Recovery means your sexuality is no longer running a compulsory, harmful pattern. You get to build a life where desire can exist without becoming an order.

Closing reframe

‘Mujhe apni sexuality se darna hai’ nahi. ‘Mujhe apne choices, consent, health aur values ko clearly lead karna hai.’

PART 06



Urge Aur Emotion Toolkit

Small skills, repeated in real moments

Skills tab kaam karti hain jab woh moment mein yaad aur accessible hon. Isliye yahan long theory ke bajay short, testable tools hain. Kisi ek tool ke fail hone ka matlab recovery fail nahi; next layer choose karo.

IN THIS PART

13 Craving Toolbox: Pause Se Next Action Tak
Wave ko observe karo, automatic action ko interrupt karo

14 Stress, Loneliness, Sleep Aur Mental Health
Underlying pressure ko treat kiye bina recovery incomplete lagti hai

CHAPTER 13

Craving Toolbox: Pause Se Next Action Tak

Wave ko observe karo, automatic action ko interrupt karo

RESEARCH TRAIL R4 · R5 · R10 · R11 · R17 · R21 · R22 · R23 · R24

Urge ka clock nahi, process dekho

Log aksar ‘craving exactly 90 seconds ya ten minutes mein khatam hoti hai’ jaise fixed rules quote karte hain. Real experience variable hai: kuch waves seconds mein shift hoti hain, kuch return karti hain, kuch withdrawal/mental-health condition ke saath longer rehti hain. Timer useful ho sakta hai, lekin guarantee nahi. Better goal: urge ko notice karna, unsafe action postpone karna, support add karna, and next review karna — regardless of exact duration.

Craving intensity 0–10 se note karna optional hai, not compulsory. Numbers par obsess na karo. Urge high ho toh simple action; moderate ho toh thought record; low ho toh prevention. If severe withdrawal or medical symptoms accompany craving, treat it as health issue, not only a wave. ‘Surfing’ should never delay seizure, confusion, breathing difficulty or overdose response.

> **Success definition**
> Success = urge ke saath bhi safe choice ya early help. Success ≠ urge ka immediately disappear hona.

Grounding: present ko five senses se anchor karo

Feet floor par, surroundings ko name karo: five visible objects, four sounds, three physical sensations, two smells, one taste — count ko rigid mat banao, sensory attention is the point. Cool water on hands, holding a cup, feeling chair support, or stepping into fresh air can help. Choose senses comfortable for you; trauma survivors may find some body-focused practices distressing, so external grounding is an option.

Grounding ke baad immediately next safe action karo, otherwise same trigger environment mein attention wapas ja sakti hai. Example: ground → phone away → shared space → message support. Grounding ko test karo when calm, so it is not a strange new technique during crisis. If dizziness, panic or discomfort increases, stop and choose another method or professional guidance.

POCKET GROUNDING

01 Name where you are and today’s date.
02 Notice three neutral things around you.
03 Feel one supportive surface.
04 Say: ‘Main current moment mein safe next step choose kar sakta hoon.’
05 Move to your preselected next action.

Breathing: gentle, not forced

Breathing exercise ka goal big deep breaths lena nahi; comfortable, slightly slower exhale and reduced rushing ho sakta hai. Try relaxed posture, inhale normally, exhale gently without straining, for a few cycles. If you have respiratory/cardiac issues, feel dizzy, or breath focus increases panic, stop. No breath-holding challenges or hyperventilation. Grounding through sounds or movement may suit you better.

Breathing craving ko magically erase nahi karegi. It can lower physiological urgency enough to remember your next step. Pair it with a sentence: ‘Exhale; decision abhi automatic nahi.’ Then follow the environment/support plan. If breathing difficulty is new, severe, substance-linked, or accompanied by blue lips/unresponsiveness, it is not a coping exercise situation — call emergency services.

- Comfortable posture; no force.
- Normal inhale, gentle unforced exhale.
- Stop if dizzy or more panicked.
- Use as bridge to action, not entire plan.
- New/severe breathing symptoms = medical help.

Movement aur daily activity

WHO adults ke liye regular aerobic and muscle-strengthening activity recommend karta hai for general health, but recovery mein start point individual health/ability par depend karega. You do not need an intense workout to prove commitment. A brief walk, gentle stretching, household task, or clinician-approved activity can change context and reduce idle cue exposure. Persistent exhaustion, pain, dizziness, eating disorder, or medical condition ho toh personalised advice lo.

Movement ko punishment mat banao: ‘lapse hua, ab 100 push-ups’ shame loop ko reinforce karta hai. Also avoid replacing one compulsion with exercise compulsion. Goal sustainable rhythm and broader life, not exhaustion. Start below your maximum, increase gradually, hydrate as appropriate, and stop for concerning symptoms. Physical activity adjunct hai; substance-use disorder or CSBD ka stand-alone cure nahi.

GENTLE ACTIVITY MENU

01 One two-minute option for high-urge moments.
02 One ten-minute option for daily routine.
03 One social/outdoor option you value.
04 Health adaptations or clinician questions.
05 Signs to stop and seek help.

Thought record: courtroom nahi, curiosity

Thought record mein situation, automatic thought, emotion, evidence for/against, balanced alternative and next action hote hain. Aim automatic thought ko legally prove wrong karna nahi, uski certainty ko loosen karna hai. ‘Main ek lapse ke baad hopeless hoon’ ke against evidence: previous return-to-plan, current support, available appointment. Balanced thought: ‘Lapse serious ho sakta hai, lekin main abhi escalation rok sakta hoon aur help le sakta hoon.’

Avoid toxic positivity. ‘Sab perfect hai’ false hai. Balanced thought difficult reality ko acknowledge karta hai and action option include karta hai. If thoughts involve self-harm, worthlessness with inability to stay safe, paranoia, hallucinations or mania, simple thought record alone insufficient. Urgent mental-health support lo. CBT tools are helpful within the right level of care.

- Automatic thought exactly what?
- What emotion and urge followed?
- What facts support it? What facts complicate it?
- What is a balanced, believable alternative?
- What safe action follows now?

Opposite action: emotion ke opposite nahi, compulsion ke opposite

Opposite action ka matlab valid emotion ko deny karna nahi. Anger feel kar sakte ho without aggressive/use behaviour; loneliness feel kar sakte ho without compulsive escape; shame feel kar sakte ho without hiding from clinician. Action choose karo jo long-term value serve kare: urge to isolate → brief safe contact; urge to seek substance → leave access and call support; urge to scroll → device away and start defined task.

First action small enough to do while emotion is high. ‘I will rebuild my whole life today’ is not opposite action; it is overwhelm. ‘I will send one honest text’ is. If safety risk exists, opposite action may be leaving the situation, asking for urgent help, or not engaging in conflict. Your values guide the action, not a demand to feel cheerful.

> **Ask**
> ‘Agar main is feeling ko bina harmful behaviour ke honour karoon, toh next two minutes ka action kya hoga?’

Coping card ko test aur revise karo

Coping card wallet/phone mein ho sakti hai, but sensitive details avoid karo. Front: top trigger, warning thought, three actions, two contacts. Back: emergency signs and 112, values reminder, professional appointment. Keep wording brief. Test the card once in a low-stress moment: can you find it, read it, and do the first step? If not, simplify or move it.

After real use, mark what happened. Did action take too long? Contact unavailable? Location unsafe? Update. A coping plan is a prototype, not a sacred contract. Share appropriate parts with clinician/support person so they can help you use it. The card should make safe action easier under stress; anything decorative but unusable can go.

MY COPING CARD

01 Warning sign: ___
02 Permission thought: ___
03 First two-minute action: ___
04 Backup action/location: ___
05 Contacts + emergency escalation: ___
06 One values-based sentence: ___

CHAPTER 14

Stress, Loneliness, Sleep Aur Mental Health

Underlying pressure ko treat kiye bina recovery incomplete lagti hai

RESEARCH TRAIL R4 · R10 · R11 · R12 · R16 · R17 · R21 · R22 · R23 · R24

Stress ko life se delete nahi, capacity se meet karna hai

Stressful events recovery ke baad bhi aayenge. Goal zero stress nahi; predictable load, recovery breaks, support and problem-solving. Stress ko controllable vs not-currently-controllable mein divide karo. Deadline ko tasks mein break kar sakte ho; someone else's reaction ko fully control nahi. First category mein action, second mein acceptance, boundaries and support. Both categories mein old substance/compulsion route ko default mat rehne do.

Daily 'minimum viable day' banao for high-stress periods: prescribed treatment, one necessary task, food/hydration as appropriate, one support contact, basic hygiene, and safe wind-down. Extra goals temporarily reduce karo. This is not giving up; it is protecting the essentials. If stress comes with inability to function, severe panic, persistent depression, or suicidal thoughts, professional assessment is needed.

STRESS TRIAGE

01 What is within my control today?
02 What needs acceptance or boundary rather than more effort?
03 What is the one necessary task?
04 Who can share the load?
05 What is my minimum safe routine tonight?

Loneliness ko instant intimacy se solve mat karo

Loneliness ek signal ho sakta hai ki connection, belonging or being understood ki need hai. Substance/use communities or compulsive sexual behaviour briefly contact/escape de sakte hain, but not necessarily safe connection. Build a ladder: low-pressure contact, regular activity, peer/professional group, deeper friendship, therapy. One person par entire emotional load rakhna relationship ko strain kar sakta hai; multiple safe supports better.

Connection ka first step ordinary ho sakta hai: neighbour ko hello, classmate se notes, support meeting, family member ke saath tea, community volunteering. Online communities use karo with privacy and boundaries; explicit/triggering spaces avoid. If social anxiety severe hai, gradual practice and therapy help kar sakte hain. Loneliness aapse koi defect prove nahi karti; it is a human need requiring repeated safe contact.

- One low-pressure contact today.
- One recurring weekly activity.
- One professional/peer support option.
- One person for deeper conversation when ready.
- One boundary to keep online connection safe.

Boredom aur empty calendar

Early recovery mein time unexpectedly empty lag sakta hai, especially jab behaviour hours consume karta tha. Us gap ko immediately productive optimisation se bhar dena unnecessary pressure hai. Mix rest, pleasure, mastery and connection. Rest: quiet break. Pleasure: music, safe hobby. Mastery: small task. Connection: person/community. Different rewards build a life that does not rely on one intense behaviour.

Choose activities based on actual preference, not what looks impressive. If you do not know what you enjoy, experiment in short blocks without commitment. Avoid high-risk versions: bar-based socialising early in alcohol recovery, explicit/triggering entertainment in porn compulsion, or gambling/shopping as replacement compulsion. New behaviour bhi out of

control lage toh review karo. Recovery is not transferring the entire cycle to a new object.

> **Four-slot evening**
> Rest + simple meal + one enjoyable/meaningful activity + one connection/check-in. Keep it small enough to repeat.

Sleep ko support karo, perfect banane ka pressure nahi

Regular wake time, daylight/movement as appropriate, reduced late-night stimulating content, comfortable sleep environment, and device boundaries can support sleep. Caffeine/nicotine timing and other substances/medicines can affect sleep; individual advice clinician se lo. Alcohol/sedatives ko sleep aid ki tarah self-use karna risky hai, and stopping them abruptly if dependent bhi unsafe. Persistent insomnia ke liye CBT-I or medical assessment useful ho sakta hai.

One bad night ko catastrophe mat banao. ‘Ab main relapse karunga’ permission/anxiety loop create karta hai. Instead next-day plan simplify, support increase, avoid driving if sleepy/impaired, and follow professional advice. If several nights of very little sleep come with unusually high energy, impulsivity, inflated confidence, racing thoughts or sexual disinhibition, urgent mental-health assessment chahiye; it may be more than ordinary insomnia.

- Keep one consistent wake-time anchor where possible.
- Create a low-stimulation wind-down.
- No self-prescribed sleep substances or abrupt medication changes.
- Reduce next-day risks after poor sleep.
- Seek help for persistent, severe or unusual sleep/mood changes.

Co-occurring mental-health concerns

Depression, anxiety, PTSD, OCD, ADHD, bipolar disorder, psychosis, eating disorders and other conditions can interact with substance use or compulsive sexual behaviour. Sometimes substance/behaviour is coping; sometimes it worsens symptoms; sometimes both cycles reinforce each other. Sequential ‘pehle addiction perfectly cure, phir mental health’ often unhelpful; integrated assessment better. Clinician ko full symptoms and timeline batao.

Warning signs needing prompt help: persistent inability to function, severe panic, hallucinations, paranoia, mania-like reduced sleep/high energy, self-harm, suicidal thoughts, or risk to others. Tele-MANAS 14416 / 1800-891-4416 support/referral ke liye; immediate danger 112. Emergency services and qualified care ko book exercises se replace mat karo. Treatment for mental health is not a distraction from recovery; often it is recovery infrastructure.

> **Say it early**
> Clinician ko suicidal thoughts, self-harm, hallucinations or risk to another person batana urgent safety information hai. Hide karna recovery ko safer nahi banata.

Self-compassion as daily practice

Self-compassion ka simple format: acknowledge difficulty, remember shared humanity, choose responsible kindness. ‘Yeh craving difficult hai; aisa experience many people have; main ab safe step loonga.’ Yeh excuse nahi because safe action included. Avoid global labels. Speak to yourself as you would to someone you are responsibly helping: honest, firm, non-humiliating.

Daily practice can be one sentence after a difficult moment, plus one repair action. If you harmed someone, compassion does not erase accountability or their boundaries. If you feel urges you dislike, you need not hate yourself for having them; actions and support are where responsibility operates. Over time, reduced shame can make honesty and help-seeking easier.

THREE-LINE RESET

01 Difficulty: ‘Is moment mein ___ difficult hai.’
02 Common humanity: ‘Struggle ka matlab main alone/defective nahi.’
03 Responsible action: ‘Main ab ___ karunga, aur zaroorat ho toh ___ se help lunga.’

PART 07



Lapse, Repair Aur Support

Slip ko data banao, disaster identity nahi

Recovery mein setbacks possible hain, especially early stages aur high-stress transitions mein. Unhe minimise bhi nahi karna, catastrophe bhi nahi banana. Correct response: immediate safety, honest information, early support, functional analysis and revised plan.

IN THIS PART

15 Lapse Ke Baad: First Hour Se Next Week Tak
Escalation rokna aur treatment mein wapas aana

16 Support System Aur Real-Life Repair
Recovery ko relationships, work aur daily stability mein translate karo

CHAPTER 15

Lapse Ke Baad: First Hour Se Next Week Tak

Escalation rokna aur treatment mein wapas aana

RESEARCH TRAIL R10 · R11 · R12 · R13 · R14 · R16 · R17 · R21 · R22 · R24

Lapse aur relapse ko useful language do

Lapse ko single return episode, relapse ko broader return to problematic pattern ki tarah use kiya ja sakta hai, but clinical definitions vary. Important yeh hai ki single event ke baad ‘ab sab khatam’ bolna additional risk create karta hai. Ek lapse ko ignore karna bhi wrong hai kyunki medical risk, agreement breach or escalating control loss involved ho sakta hai. Balanced response: ‘Yeh significant event hai; main ab escalation rokne aur plan revise karne ke steps loonga.’

Avoid counter reset drama. Agar counter meaningful hai, update karo privately and continue. But recovery actions ko zero mat karo: therapy attended, honest call, emergency avoided, support used — yeh still count. The person who seeks help after a lapse is doing recovery behaviour. The lapse needs analysis; your identity does not need sentencing.

> **Middle path**
> Na ‘koi baat nahi’ na ‘main hopeless hoon.’ Better: ‘Iske risks hain; main immediate safe action aur early review choose kar raha hoon.’

First hour: safety before analysis

Current intoxication/withdrawal, breathing, consciousness, injury, chest symptoms, seizure, severe confusion, hallucinations, agitation, suicidal thinking or another person’s safety assess karo. Red flag = 112/emergency. Do not drive, swim, operate machinery, or stay alone if unsafe. Stop additional access where safe and move to a trusted, non-using person/environment. Do not take random medication or another substance to ‘balance’ effects.

Compulsive masturbation/porn lapse mein physical injury, non-consensual behaviour, illegal content, financial risk, exposure/privacy threat or self-harm thoughts ho toh respective urgent professional/legal/emergency support priority. Otherwise device/content se distance, shared safe space, and contact support. Do not immediately write a long graphic post or disclose publicly from shame. First stabilise, then decide what needs to be shared and with whom.

- Stop escalation and move away from access if safe.
- Check emergency signs; call 112 when present.
- No driving, mixing, self-medication or punishment.
- Contact trusted/professional support early.
- Protect other people’s safety, consent and privacy.

Substance lapse: tolerance and medication caution

After reduced use/abstinence, tolerance may be lower, especially opioids. Returning to previous amounts can increase overdose risk. Alcohol/sedatives/opioids combinations further complicate risk. If unusual sleepiness, slowed breathing, unresponsiveness or other emergency signs appear, immediate emergency help. Even if you feel okay, contact your treatment provider early for next steps, especially if maintenance medication or naltrexone/disulfiram/sedatives are involved.

Do not restart, stop or double prescribed medication based on internet instructions. Prescriber needs substance, amount estimate, last use, current symptoms and medication timing. This is not about getting scolded; it prevents dangerous interactions. A lapse can also be a sign current level of care insufficient; discuss more frequent visits, medication review, intensive programme or safer living environment if needed.

> **Same-day principle**
> Non-emergency substance lapse ho toh bhi same-day/earliest clinical or programme contact safer hai than waiting for next month’s appointment.

Compulsion lapse: shame-binge chain ko break karo

After a lapse, mind may say ‘counter toot gaya, ab aur kar lo.’ This is permission thought, not logical conclusion. Additional episodes can increase lost time, injury, financial/relationship harm and shame. Choose a stopping ritual that is not punitive: stand up, wash face, device away, change location, drink water if comfortable, contact support, and do one ordinary task. No starvation, excessive exercise, self-harm, or humiliating confession.

If partner agreement/health/finances are affected, appropriate honest conversation or professional guidance matters. But do not use impulsive graphic disclosure as self-punishment. Plan calm conversation, relevant facts, responsibility and repair. If another person’s consent/safety was violated, their protection is priority and specialist help is necessary. A lapse is not permission to continue; it is a cue to re-enter support quickly.

COMPULSION RESET

01 Device/content/access se distance.
02 Safe location + non-punitive body reset.
03 One support/professional contact if risk or repeated pattern.
04 Protect health, consent, finances and privacy.
05 Schedule functional analysis after stability.

Functional analysis: ‘kyun’ se zyada ‘kaise’

‘Main aisa kyun hoon?’ often shame spiral create karta hai. Functional analysis asks ‘Kaise sequence bani?’ Start before the episode: vulnerability factors (sleep, conflict, stress), trigger, permission thought, access, first action, failed barrier, immediate result, later consequence. Identify earliest modifiable link and one missing support. Avoid turning analysis into exhaustive explicit/detail recall, which can itself trigger.

End with one change only? Sometimes one is enough; sometimes medical risk requires multiple immediate steps. Prioritise high-impact: emergency follow-up, remove access, move device, contact clinician, avoid specific event, repair sleep. Then test. If same failure repeats, increase support rather than adding ten complicated rules. A simpler plan used is better than an impressive plan avoided.

- What made me vulnerable before the trigger?
- What was the exact permission thought?
- Which barrier was missing or bypassed?
- What immediate relief reinforced the cycle?
- What one high-impact change and support increase now?

Repair and re-entry within 24–72 hours

After stabilisation, re-enter routine: prescribed treatment, appointment, food/rest, safe social contact, and one meaningful task. Repair practical damage: missed work message, financial safeguards, sleep catch-up plan (without unsafe medication), device reset, or relationship boundary. Do not attempt every life repair in one day. Choose what prevents further harm first.

Within next week, review with clinician/therapist/support: was lapse isolated or return to pattern? Any new risks? Current treatment adequate? Which early warning signs appeared? What is revised if–then plan? If no lapse occurred but near-miss happened, same review useful. Near-misses are free data — not something to hide because 'technically nothing happened.' Early learning can prevent a full episode.

72-HOUR RE-ENTRY

01 Medical/mental-health safety addressed.
02 Treatment/support contact completed or scheduled.
03 Immediate practical harm contained.
04 One routine and one values-based action resumed.
05 Written plan revision and next review date.

CHAPTER 16

Support System Aur Real-Life Repair

Recovery ko relationships, work aur daily stability mein translate karo

RESEARCH TRAIL R10 · R11 · R12 · R16 · R19 · R20 · R21 · R22 · R25 · R26

Safe support person kaise pehchanein

Safe person necessarily closest person nahi hota. Qualities: respects privacy, does not shame, can stay calm, follows agreed role, does not offer substances or trigger content, and knows when to escalate. They need not be available 24/7. Better two or three supports with clear roles than one overwhelmed person. A professional, peer, friend, family member and helpline can each serve different functions.

Before crisis, ask consent: ‘Kya main difficult moment mein call kar sakta hoon? Agar unavailable ho toh main second contact use karunga.’ Define what helps: listen, stay on phone, accompany appointment, remove access per plan, or call emergency. Define what does not: interrogation, graphic details, public disclosure, medical advice. Support person can say no or change availability; respect that boundary.

- Role and availability are explicit.
- Privacy and non-shaming communication.
- No substance/behaviour enabling or triggering.
- Emergency signs and escalation known.
- Backup contacts so no single person is overloaded.

Family ke liye do / don’t

Family often fear, anger, guilt or exhaustion feel karti hai. Those feelings deserve support too, but behaviour towards the person must remain safe. Do: learn evidence-based information, encourage assessment, help logistics, maintain calm boundaries, notice effort, seek family counselling, and plan emergencies. Don’t: hit, restrain, lock up, publicly shame, secretly stop medicines, force detox, threaten abandonment to control, or demand immediate trust.

Family boundaries are valid: no money for substances, no using in home, no abusive behaviour, no driving children while impaired, no shared-device explicit content. Boundary should be specific and enforceable, not a rage-based threat. If violence/coercive control exists, safety planning and specialist services take priority over joint recovery meetings. Recovery does not require family to tolerate harm.

> **Family sentence**
> ‘Hum tumhari dignity respect karenge aur treatment support karenge; saath hi home mein safety boundaries clear rahengi. Dono cheezein saath possible hain.’

Accountability without surveillance

Healthy accountability answers: what behaviour, how often reviewed, what information shared, and what action after concern. Example: weekly fifteen-minute check-in on appointments, high-risk moments, plan use and safety. Not hourly location reports or unrestricted device search unless exceptional, consensual, professionally guided safety context — and even then regularly reviewed. Accountability should build internal control, not permanent external dependence.

Use measurable actions, not loyalty tests. ‘Did you attend appointment?’ is clearer than ‘Prove you care.’ ‘Did you follow agreed financial safeguard?’ is clearer than ‘Show every private message.’ Privacy remains important; accountability does not mean sharing every sexual thought. If a lapse violates relationship agreements, relevant facts and repair are necessary, but graphic interrogation rarely helps.

ACCOUNTABILITY AGREEMENT

01 Purpose: safety/recovery, not punishment.
02 Specific behaviours and review frequency.
03 Minimum necessary information.
04 Roles and backup if check-in missed.
05 Review date to reduce/adjust monitoring as stability grows.

Work, study aur money repair

Recovery mein practical stability strong protective factor ho sakti hai. Work/study mein start with attendance, essential tasks and honest but appropriately bounded communication. You need not disclose full medical/sexual history to everyone; HR, doctor, counsellor or designated authority may be more appropriate depending on context. Ask for legally/policy-available leave or accommodations through proper channels if needed. Avoid making major career decisions while intoxicated or in acute crisis.

Money repair: list essential expenses, debts/obligations, risky spending routes, and agreed safeguards. If shared finances affected, transparent relevant information and professional advice may be needed. Do not hand over all autonomy indefinitely without review; use time-limited safeguards. For illegal debt, exploitation, threats or financial abuse, seek qualified legal/safety support. Financial recovery can be gradual; the goal is stopping further harm and building predictable routines.

- Stabilise attendance and essential responsibilities.
- Disclose only necessary information to appropriate people.
- Protect essentials and shared financial obligations.
- Use consensual, reviewable spending safeguards.
- Get legal/safety help for threats or exploitation.

Support map: four layers

Layer one: immediate safety — 112, emergency department, trusted nearby person. Layer two: professional — physician, psychiatrist, therapist, de-addiction service, sexual-health clinician. Layer three: ongoing personal/peer support — family/friend/group. Layer four: everyday structure — work/school, sleep routine, activities, community. Different layers solve different problems; friends cannot replace emergency care, and professionals cannot be your entire social life.

Map current gaps. Perhaps you have friends but no clinician; therapy but no emergency plan; family but unsafe communication; information but no daily routine. Choose one gap to fill this week. Save helplines: drug-de-addiction 14446, Tele-MANAS 14416 / 1800-891-4416, tobacco quitline 1800-11-2356, emergency 112. Availability/services can change; verify through official channels when needed.

MY FOUR-LAYER MAP

01 Emergency: who/where/how?
02 Professional: name, service, next appointment.
03 Personal/peer: two consented contacts and roles.
04 Structure: one daily and one weekly stabilising activity.
05 Biggest current gap and next action.

When home or relationship is unsafe

If someone threatens, hits, controls money/movement, forces sexual activity, intentionally supplies substances, or punishes recovery attempts, standard communication exercises may not be enough. Your safety and any children/dependents' safety come first. Reach out to trusted safe person, appropriate emergency/legal/domestic-violence service or healthcare professional. Avoid confronting a dangerous person alone or announcing plans if that increases risk.

Digital safety bhi matter karti hai: shared devices/accounts may expose help-seeking. Use a safer device where possible and follow specialist guidance; do not create a detailed visible plan that a controlling person could use against you. India emergency 112 for immediate danger. Recovery support should never ask you to remain in danger as proof of commitment. Humane, safe care is the standard.

> **Safety first**
> Agar recovery plan ka koi step violence or coercion trigger karta hai, pause and seek specialist safety support. Do not treat danger as a motivation exercise.

PART 08



Recovery Ko Life Mein Convert Karo

Thirty days ka start, ninety days ka rhythm, long-term choice

Recovery ka goal har second urge ke baare mein sochna nahi. Goal hai aisi daily life jismein health, relationships, work, meaning and support itne real hon ki old pattern ko har decision ka centre na banna pade. Yeh part sustainable rhythm aur 30-day workbook ka safe use explain karta hai.

IN THIS PART

17 Ninety-Day Rhythm Aur Long-Term Maintenance
Identity ko intensity se nahi, repeated actions se banao

18 30-Day Workbook Kaise Use Karein
Structure do, perfection nahi

CHAPTER 17

Ninety-Day Rhythm Aur Long-Term Maintenance

Identity ko intensity se nahi, repeated actions se banao

RESEARCH TRAIL R10 · R11 · R12 · R14 · R17 · R18 · R23 · R24 · R26

Identity ko behaviour ke chhote proofs do

'Main ab completely new person hoon' excitement de sakta hai, but fragile bhi. Better identity statements observable: 'Main early help maangta hoon,' 'Main high-risk situation mein pause karta hoon,' 'Main prescribed treatment follow karta hoon,' 'Main consent aur agreements respect karta hoon.' Har small action identity ko proof deta hai. Lapse proof ko erase nahi karta; it signals need for more support.

Avoid recovery becoming your only identity. You are also a friend, learner, worker, creator, parent, partner, community member. Meaningful roles reduce the space available for compulsive pattern and make life rewarding. Do not overload yourself with big achievements to prove worth. Choose one role each week and one action that expresses it. Stable identity grows quietly.

IDENTITY EVIDENCE

01 I am a person who ___
02 This week's smallest proof: ___
03 A non-recovery role I value: ___
04 One action expressing that role: ___
05 Support I will use if proof feels difficult: ___

Four weekly anchors

Anchor one: health/treatment — appointments, prescribed plan, sleep/meals/movement as appropriate. Anchor two: connection — one meaningful contact and one recurring group/activity. Anchor three: responsibility — work/study/home essentials. Anchor four: restoration — enjoyable, calming or spiritual practice aligned with your values, without becoming compulsory. These anchors create rhythm even when motivation fluctuates.

Weekly schedule ko overcrowded mat banao. Put anchors at realistic times and leave transition space. Early recovery mein fewer commitments with support better than ambitious collapse. Review health needs with clinician before exercise or major routine changes. If a week goes off track, restore the anchor with highest safety impact first: treatment and immediate safety, then connection, responsibility, restoration.

- Health/treatment anchor: what and when?
- Connection anchor: who/where?
- Responsibility anchor: one essential commitment.
- Restoration anchor: one safe activity.
- Backup version for a difficult week.

Rewards ko healthy aur flexible rakho

Behaviour change ke liye healthy rewards useful hain: a meal you enjoy, time for hobby, a small purchase within budget, social outing, or acknowledging progress. Reward should not recreate the old risk, financial harm or another compulsion. Avoid using sexual access, substances, gambling or excessive spending as reward. Also avoid making basic needs conditional on perfect behaviour.

Contingency management clinical settings mein structured reinforcement use karta hai, especially stimulant-use disorders mein. Home version simple and non-punitive ho sakta hai: appointment attend → planned enjoyable activity; week of support check-ins → shared outing. Do not withhold medicine, food, affection, or essential care for missed targets. Reward actions, not only abstinence, so effort remains visible even when outcome imperfect.

Good reward test
Safe? Affordable? Values-aligned? Not another compulsive cycle? Not controlled as humiliation by someone else? Then it may be useful.

Transitions ko high-risk maano, weakness nahi

New job, exams, travel, festival, wedding, breakup, grief, illness, financial change, moving home, relationship shift, or medication/health change can disrupt routine. Even positive transitions increase decision load and access to new cues. Before a known transition, update the map: where will I stay, who is support, what access changes, what sleep/routine changes, what appointments continue?

Travel ke liye prescribed medicines original packaging/documentation and local rules clinician/pharmacist se confirm karo; do not improvise. Digital compulsion mein hotel privacy, idle time, new device or unrestricted network risk badha sakta hai. Social events mein substance offers and consent risks plan karo. Increase support temporarily. Transition plan is not pessimism; it protects the progress you have built.

TRANSITION CARD
01 What changes in place, people, time and access?
02 What remains stable?
03 Which support increases temporarily?
04 What is my exit/backup plan?
05 Which early warning sign means professional contact?

Early warning signs ko personal banao

Generic list ke saath apne early signs identify karo: stopping appointments, secrecy, sleep change, irritability, romanticising old behaviour, reconnecting risky contacts, bypassing blockers, hiding spending, skipping meals, withdrawing from safe people, or ‘main ab cured hoon, support ki zaroorat nahi.’ One sign does not prove relapse; several together deserve review.

Create three zones. Green: stable, routine working. Amber: early signs, increase contact and reduce access. Red: lapse, medical/mental-health

risk, or danger to others — urgent professional/emergency response. Write actions for each zone in advance. Share appropriate version with support person so they can respond without guessing. Review zones after every significant event.

- GREEN: maintain routine and scheduled review.
- AMBER: contact support, simplify week, reinforce barriers, consider clinician review.
- RED: immediate safety, urgent professional/emergency help, no solo management.
- After RED: re-entry and plan revision, not punishment.

Helping others without losing your own footing

Recovery ke baad others ko help karna meaningful ho sakta hai, but too early over-responsibility stress create kar sakti hai. Share resources and your experience without claiming universal cure or acting as doctor/therapist. Do not collect others' graphic histories, promise confidentiality you cannot maintain, or manage emergencies alone. Point to qualified services and protect your own routines.

Peer support mein boundaries: time limits, no loans/procurement, no romantic/sexual exploitation, no medication sharing, no public disclosure. If another person is in danger, involve emergency/professional support. Your recovery does not require becoming a full-time rescuer. Sustainable helping is respectful, bounded and evidence-aware.

> **Useful help**
> 'Main yeh verified resource share kar sakta hoon aur appointment tak support kar sakta hoon' is safer than 'Main tumhe cure kar dunga.'

CHAPTER 18

30-Day Workbook Kaise Use Karein

Structure do, perfection nahi

RESEARCH TRAIL R1 · R3 · R4 · R10 · R11 · R13 · R14 · R16 · R21 · R22 · R24

Day zero: safety gate

Workbook start karne se pehle decide karo ki self-guided exercises appropriate hain ya professional assessment first. Heavy alcohol/sedative use, previous severe withdrawal, opioid/mixed use, current severe symptoms, pregnancy/serious illness, self-harm/suicidal thoughts, violence/non-consensual risk, or inability to stay safe ho toh workbook ko treatment replacement mat banao. Contact appropriate service; exercises can support care after safety planning.

Substance aur compulsion dono patterns hon toh sab kuch simultaneously perfect karne ka goal mat rakho. Highest safety risk primary track banega. Aap dono ke liye notes rakh sakte ho, but one main behaviour target at a time often more manageable. Clinician se priorities align karo. Day one perfect abstinence promise nahi; accurate baseline and safe next step hai.

> **Do not delay care**
> Workbook completion ke liye doctor appointment postpone mat karo. Emergency signs mein page band karo aur help lo.

Track S, Track M, ya combined support

Track S substance-use recovery ke liye hai: medical/treatment plan, triggers, access, support, craving, lapse safety. **Track M** compulsive masturbation/porn pattern ke liye: values-based goal, impaired control, digital/emotional cues, consent/relationships, professional review. Dono tracks common skills share karte hain, but daily prompt mein apna relevant version choose karo.

Combined pattern ho toh substance use ke time consent risk, sexual behaviour and overdose/withdrawal safety interconnected ho sakte hain. Professional assessment important. Workbook mein dono prompts complete karna compulsory nahi; primary risk pehle. Avoid using one behaviour to manage withdrawal or the other compulsion. Integrated treatment can address both without setting them against each other.

- Track S: follow medical/treatment instructions before self-help.
- Track M: no diagnosis from frequency or moral guilt alone.
- Combined: choose highest safety priority; disclose both to clinician.
- Every day: one action and one honest reflection is enough.

Daily page ka rhythm

Morning mein focus and one action read karo. Day mein relevant prompt apply karo. Evening mein five-minute reflection: kya hua, kya help hua, kya adjust karna hai. Notes short rakh sakte ho; pages par graphic details, illegal procurement information or someone else’s private data likhne ki zaroorat nahi. If keeping the book/device creates privacy risk, store securely or use clinician-approved alternative.

Missed day = resume next day. Do not double exercises as punishment. If day’s action unrealistic, scale down: one contact instead of three, two-minute walk instead of workout, five lines instead of full page. Consistency with flexibility beats intensity with collapse. Mark ‘not applicable’ where needed; not every prompt fits every person.

DAILY MINIMUM

01 Read today’s safety/focus line.
02 Do one relevant action.
03 Record one fact and one learning.
04 Choose tomorrow’s one adjustment.
05 Escalate if safety changed.

Weekly review: evidence, not mood only

Days 7, 14, 21 and 30 par review karo. Mood fluctuates; evidence wider dekho. Kitne support contacts, appointments, pauses, safer choices, routine anchors, early warnings noticed? Any medical/relationship/financial risk? What improved in sleep, work, control, honesty? If nothing improved, that is also data — current plan/support may be inadequate, not proof you are hopeless.

Review ke baad one keep, one change, one ask. Keep the useful action. Change one weak barrier. Ask for specific help. If repeated lapses or worsening symptoms, increase professional level of care rather than simply repeating workbook. Thirty days is a starting container, not biological reset or guaranteed cure.

- KEEP: which action earned continuation?
- CHANGE: which plan element did not fit real life?
- ASK: what professional/personal support is needed?
- ESCALATE: any safety issue requiring more than self-help?

Progress score without obsession

Optional 0–2 rating: 0 = not done, 1 = attempted/partly done, 2 = completed. Rating effort and skill use ko capture karta hai, not moral worth. Do not total it into ‘good/bad person’ score. Weekly maximum/minimum chase mat karo. If numbers create anxiety or compulsive checking, stop scoring and use one sentence: ‘Aaj maine recovery ke liye ___ kiya.’

Urges and behaviour counts can be clinically useful in some plans, but if they trigger shame or obsessive monitoring, clinician ke saath method choose karo. Functioning measures often more informative: attended work, slept safely, no risky situation, honest conversation, treatment adherence. Your workbook should serve your recovery; it must not become another ritual you have to perfectly perform.

> **Permission**
> Aap blank space chhod sakte ho, prompt modify kar sakte ho, professional ke saath skip kar sakte ho. Safety and usefulness > completion.

Day 30 ke baad

Day 30 par decide karo: continue current rhythm, repeat a useful week, deepen therapy, or increase level of care. Do not automatically stop support because workbook finished. Build next 30 days with four weekly anchors and known high-risk dates. Schedule follow-up before motivation drops. Keep the lapse plan accessible.

If the month was stable, acknowledge actions and people who helped. If it was difficult, acknowledge honesty and identify needed support. Both are legitimate starting points for next month. Long-term recovery is not a graduation certificate; it is continuing care and a life you do not need to escape repeatedly.

NEXT-MONTH COMMITMENT

01 My highest-value routine to continue.
02 My biggest unresolved risk.
03 Professional/support review date.
04 One meaningful life goal beyond recovery.
05 My early-warning and emergency plan location.

WORKBOOK



Your 30-Day Recovery Workbook

One page a day • Track S + Track M • No perfection requirement

Yeh thirty-day structure treatment replacement nahi, treatment aur daily action ka companion hai. Har din common practice ke saath substance aur compulsion tracks ke specific prompts hain. Apna relevant track choose karo. Missed day ko skip nahi, simply resume karo — doubling ya punishment nahi.

DAY 01 / 30 • WEEK 1

Baseline Without Judgement

Aaj perfect result nahi, accurate starting point.

Apne target pattern ko non-graphic, observable words mein likho: kya behaviour, kitne time se, kin situations mein, aur concrete impact kya. Sirf frequency mat likho; control, health, sleep, work, money, relationships aur safety include karo. Agar exact facts unsure hain, estimate label karo.

TRACK S • SUBSTANCE RECOVERY
All substances/medicines, approximate amount, last use, previous withdrawal/overdose aur current red flags list karo. Heavy alcohol/sedative/mixed use mein abrupt change nahi — professional assessment ko today's action banao.

TRACK M • MASTURBATION / PORN COMPULSION
Frequency ko diagnosis mat banao. Impaired control, persistence, distress/impairment, moral guilt vs concrete harm, and any pain/injury/consent risk separately note karo.

EVENING REFLECTION

- Aaj ka sabse important fact kya hai?
- Kya mujhe self-help se pehle professional/safety help chahiye?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 02 / 30 • WEEK 1

Safety Card

Emergency plan ko crisis se pehle ready rakho.

Ek card par 112, nearest emergency facility, Tele-MANAS 14416 / 1800-891-4416, trusted contact aur relevant helpline likho. Emergency signs short rakho: seizure, severe confusion, hallucinations, unresponsiveness, breathing problem, chest pain, suicidal/violent risk.

TRACK S • SUBSTANCE RECOVERY
National Drug-de-Addiction Helpline 14446 aur tobacco ke liye 1800-11-2356 add karo where relevant. Current medicines/allergies/conditions clinician-ready list mein rakho; public lock-screen par sensitive history nahi.

TRACK M • MASTURBATION / PORN COMPULSION
Injury, non-consensual/illegal risk, self-harm or another person in danger ka escalation clear karo. Routine urge aur emergency ko same response mat do.

EVENING REFLECTION

- Kaunsa sign par main wait nahi karunga?
- Safety card kahan accessible aur private rahega?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 03 / 30 • WEEK 1

One Honest Support Request

Help lena recovery action hai.

Ek safe person/service choose karo aur specific request bhejo: listening, appointment, check-in, transport, ya emergency role. Graphic details necessary nahi. Unki availability aur boundaries poochho; backup contact bhi rakho.

TRACK S • SUBSTANCE RECOVERY
Agar treatment appointment pending hai, aaj call/book karo. Support person ko batao ki unhe medical advice nahi deni; honest information, logistics and emergency recognition useful hain.

TRACK M • MASTURBATION / PORN COMPULSION
Request ho sakti hai: ‘Kya hum weekly non-judgemental check-in kar sakte hain? Main tumse monitoring ya graphic questioning nahi maang raha.’ Consent-based support choose karo.

EVENING REFLECTION

- Maine exactly kya request ki?
- Response kya tha aur backup kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 04 / 30 • WEEK 1

Trigger Chain Mapping

Last action se pehle ke links ko visible banao.

Ek recent episode/strong urge ko backwards trace karo: vulnerability → trigger → permission thought → access → first action → result → later cost. Shame words hatao. Earliest controllable link par circle karo.

TRACK S • SUBSTANCE RECOVERY
Place, people, money, time, withdrawal/stress aur substance access include karo. Withdrawal symptoms ko sirf emotional trigger na samjho; treatment review needed ho sakta hai.

TRACK M • MASTURBATION / PORN COMPULSION
Device, privacy, late night, scrolling, stress/loneliness, attention shift aur digital barriers include karo. Explicit content/detail recreate karna zaroori nahi.

EVENING REFLECTION

- Earliest modifiable link kaunsa hai?
- Us link par kaunsa two-minute change possible hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 05 / 30 • WEEK 1

Values-Based Goal

Goal fear se nahi, life direction se.

Complete sentence: ‘Main ___ change karna chahta hoon kyunki ___ meri life mein important hai.’ Goal specific boundary + support + review contain kare. ‘Kabhi urge nahi hogi’ goal nahi; ‘urge par safe response’ goal hai.

TRACK S • SUBSTANCE RECOVERY
Abstinence/reduction/taper goal clinician ke plan se align karo. Apne aap medication/withdrawal schedule mat set karo. Health, safety and treatment adherence ko outcome mein include karo.

TRACK M • MASTURBATION / PORN COMPULSION
Abstinence, porn-free boundary, controlled behaviour, or functioning-first goal ho sakta hai. Moral guilt alone se diagnosis nahi; choice, consent, health and real-life impact se goal refine karo.

EVENING REFLECTION

- Mera goal kis value ko protect karta hai?
- Progress ko streak ke alawa kaise dekhunga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 06 / 30 • WEEK 1

Access Aur Friction

Risky action slower, safe action easier.

One high-risk access point choose karo aur safe barrier add karo; one healthy option ko easier banao. Barrier ko test karo: kya real chain target hota hai, ya sirf decoration? Consent, privacy and dignity maintain karo.

TRACK S • SUBSTANCE RECOVERY
Substance stock/medicine changes medical plan ke saath; heavy alcohol/sedative dependence mein secret removal/abrupt stop unsafe. Safe storage/disposal and support person roles clear karo.

TRACK M • MASTURBATION / PORN COMPULSION
Device outside room, blockers, recommendation cleanup, no saved payment, shared-space use relevant ho sakte hain. Blocker = tool, cure nahi; bypass plan bhi likho.

EVENING REFLECTION

- Kaunsa barrier actual chain ko slow karega?
- Agar barrier bypass ho, mera safe reset kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 07 / 30 • WEEK 1

Week One Review

Evidence dekho, mood ka verdict nahi.

Week ke facts review karo: support contacts, appointments, safer choices, pauses, routine, risks, any lapse/near-miss. Ek KEEP, ek CHANGE, ek ASK choose karo. Missed days double mat karo.

TRACK S • SUBSTANCE RECOVERY
Any withdrawal, side effects, use/lapse, medication concern ya overdose risk ho toh clinician contact priority. Plan ko independently intensify mat karo.

TRACK M • MASTURBATION / PORN COMPULSION
Control, impairment, sleep/work, consent/relationship impact dekho — sirf count nahi. Repeated loss of control ya distress ho toh therapist/qualified assessment add karo.

EVENING REFLECTION

- KEEP / CHANGE / ASK kya hai?
- Kya support level increase karna chahiye?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 08 / 30 • WEEK 2

Permission Thoughts

Thought ko fact aur command se alag karo.

Apne top three permission thoughts likho: ‘bas ek baar’, ‘deserve karta hoon’, ‘ab kya farak’. Har ek ke saamne short, believable response aur action. Long debate nahi; label → response → move.

TRACK S • SUBSTANCE RECOVERY
‘Detox ke baad same amount safe hoga’ ya ‘medicine se ab kuch nahi hoga’ dangerous assumptions ho sakte hain. Reduced tolerance and interaction risk ko clinician/emergency plan ke saath address karo.

TRACK M • MASTURBATION / PORN COMPULSION
‘Counter toot gaya, ab binge’ ya ‘thought aaya matlab action inevitable’ ko label karo. Thought/fantasy ka hona consent violation ya action nahi; behaviour par choice and responsibility apply hoti hai.

EVENING REFLECTION

- Kaunsa thought sabse convincing lagta hai?
- Mera one-line response aur next action kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 09 / 30 • WEEK 2

Three If–Then Plans

Decision pressure se pehle rehearse karo.

Teen high-risk situations ke liye AGAR–TOH plans likho. Har plan two-minute start, backup and contact contain kare. Emergency signs ke liye alag red plan: 112/emergency, coping exercise nahi.

TRACK S • SUBSTANCE RECOVERY
Offer → short refusal → exit → support. Withdrawal symptoms → prescribed/emergency instructions. Lapse → no extra use/self-medication → same-day professional contact.

TRACK M • MASTURBATION / PORN COMPULSION
Late-night private device urge → device away → location change → safe action. Blocker bypass → stop escalation → support/therapist review. Consent risk → distance + specialist help.

EVENING REFLECTION

- Kaunsa plan realistically follow hoga?
- Backup kaunsa hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 10 / 30 • WEEK 2

Grounding Practice

Calm moment mein skill rehearse karo.

Do-minute external grounding practise karo: place/date, neutral sights/sounds, supportive surface, then safe action. Agar body focus uncomfortable ho toh external senses use karo. Dizziness/panic badhe toh stop and switch.

TRACK S • SUBSTANCE RECOVERY
Grounding craving ke saath help kar sakti hai, but severe withdrawal/overdose symptoms mein delay mat karo. Practice ke baad apna treatment/support next step complete karo.

TRACK M • MASTURBATION / PORN COMPULSION
Grounding ke baad device/location change add karo, warna same screen par attention wapas ja sakti hai. Isse thought-suppression ritual mat banao; flexible use karo.

EVENING REFLECTION

- Kaunsi sensory method comfortable thi?
- Grounding ke baad mera next action kya hoga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 11 / 30 • WEEK 2

Delay With A Real Action

Timer nahi, context change.

Ek delay plan practise karo: stand up, access/device away, location change, two-minute safe action, then review. Exact urge duration ki expectation mat rakho. Goal urge ko instantly kill karna nahi, automatic action ko postpone karna hai.

TRACK S • SUBSTANCE RECOVERY
Craving ke saath access se distance and contact. If substance already used, no more to 'balance', no driving, assess emergency signs and contact support/clinician.

TRACK M • MASTURBATION / PORN COMPULSION
Same room/same screen par countdown mat karo. Non-triggering task, shared space or support text choose karo. Repeated failure = stronger support, not self-hatred.

EVENING REFLECTION

- Delay ko real banane wala action kya tha?
- Next time kya easier hoga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 12 / 30 • WEEK 2

Alternative Relief Menu

Need ko naam do, response options badhao.

Physical, sensory, social, cognitive and practical categories mein one-one safe option likho. Underlying need identify karo: rest, comfort, connection, achievement, autonomy. Old behaviour jitna instant relief na mile, tab bhi safe action useful ho sakta hai.

TRACK S • SUBSTANCE RECOVERY
Withdrawal ko alternative relief se treat karne ki koshish nahi. Medical plan ke saath low-demand routine, support and safe environment add karo.

TRACK M • MASTURBATION / PORN COMPULSION
Porn/explicit content ko distraction menu mein nahi. Loneliness → contact, stress → task support, exhaustion → sleep routine. Persistent mental-health needs professional care deserve karte hain.

EVENING REFLECTION

- Aaj ka dominant need kya tha?
- Kaunsa response genuinely helpful tha?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 13 / 30 • WEEK 2

Gentle Movement

Activity ko support banao, punishment nahi.

Health/ability ke hisaab se brief movement choose karo: walk, stretch, household task, or clinician-approved option. Start below maximum. Dizziness, chest symptoms, severe pain or unusual shortness of breath par stop and seek advice.

TRACK S • SUBSTANCE RECOVERY
Early withdrawal/intoxication mein exercise se symptoms mask ya risk increase ho sakta hai. Clinician guidance and rest/monitoring priority. No dehydration or extreme workout challenge.

TRACK M • MASTURBATION / PORN COMPULSION
Lapse ke baad push-ups/exercise punishment mat do. Movement ka aim context change and general health; compulsive sexual behaviour ka stand-alone cure nahi.

EVENING REFLECTION

- Kaunsa gentle option sustainable tha?
- Kya maine punishment vs support difference notice kiya?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 14 / 30 • WEEK 2

Week Two Review

Near-misses bhi valuable data hain.

Week ke near-misses, skills used, barriers and support review karo. Jo almost lapse tha usmein kya rokne mein help hua? Jo fail hua usmein support kahan late aaya? One KEEP / CHANGE / ASK.

TRACK S • SUBSTANCE RECOVERY
Use/lapse, side effects, withdrawal or adherence concern ko clinician ke saath review karo. Tolerance/medication decisions self-managed nahi.

TRACK M • MASTURBATION / PORN COMPULSION
Repeated bypass, longer episodes, distress/impairment, or obsessive tracking ho toh plan/therapist review. Counter perfection se zyada control and functioning dekho.

EVENING REFLECTION

- Near-miss ne kya sikhaya?
- Kaunsa support earlier chahiye tha?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 15 / 30 • WEEK 3

Minimum Viable Day

High-stress day mein essentials protect karo.

Aaj ka minimum routine likho: treatment, one necessary task, food/hydration as appropriate, one contact, basic hygiene, safe wind-down. Extra demands temporarily reduce karo. Minimum day giving up nahi; continuity protection hai.

TRACK S • SUBSTANCE RECOVERY
Prescribed plan and appointments non-negotiable essentials. Intoxicated/impaired driving or risky work avoid. Severe symptoms ko ‘busy schedule’ se push aside mat karo.

TRACK M • MASTURBATION / PORN COMPULSION
High stress mein device-free transition, low-demand evening and support contact. Exhaustion ko ‘weak willpower’ na bolo; sleep/mental-health review consider karo.

EVENING REFLECTION

- Aaj ka essential kya hai?
- Kaunsa extra demand safely postpone hoga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 16 / 30 • WEEK 3

Sleep Boundary

Night routine ko high-risk zone se nikalo.

One consistent wake-time anchor, low-stimulation wind-down and device boundary choose karo. One bad night ko catastrophe mat banao; next day simplify and support. Persistent insomnia professional review deserve karta hai.

TRACK S • SUBSTANCE RECOVERY
Alcohol/sedatives ko sleep aid self-use nahi; dependent ho toh abrupt stopping bhi unsafe. Medicines/caffeine/substances and sleep prescriber se discuss karo.

TRACK M • MASTURBATION / PORN COMPULSION
Phone outside bedroom or agreed charging spot, non-triggering wind-down, urge par location change. Body/sexuality ko shame nahi; specific late-night chain target karo.

EVENING REFLECTION

- Meri night boundary kya hai?
- Poor sleep ke next day safety adjustment kya hoga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 17 / 30 • WEEK 3

Connection Ladder

Loneliness ko repeated safe contact se meet karo.

Ek low-pressure connection action karo: message, tea, group, class, community, or professional peer support. One person par whole load nahi; multiple layers build karo. Privacy and consent maintain karo.

TRACK S • SUBSTANCE RECOVERY
Substance-free social option choose karo; early recovery mein high-risk venues avoid. Group medical care replace nahi karta; unsafe/shaming group red flag.

TRACK M • MASTURBATION / PORN COMPULSION
Explicit/triggering spaces ko connection mat samjho. Non-graphic supportive conversation enough. Partner ko 24/7 regulator banana nahi; broader support network build karo.

EVENING REFLECTION

- Kaunsa contact safe tha?
- Recurring connection ka next opportunity kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 18 / 30 • WEEK 3

Boredom Aur Healthy Reward

Empty time ko four types of reward do.

Rest, pleasure, mastery, connection mein se two short activities plan karo. Safe, affordable, values-aligned reward choose karo. Basic needs ko perfect behaviour ki condition mat banao.

TRACK S • SUBSTANCE RECOVERY
Substance, gambling, risky spending or old using circle reward nahi. Treatment appointment/support follow-through ko acknowledge karo, not only abstinence count.

TRACK M • MASTURBATION / PORN COMPULSION
Sexual access, explicit content or partner obligation reward nahi. Partner affection/sex consent-based hai, recovery prize nahi. New compulsion develop ho toh review karo.

EVENING REFLECTION

- Kaunsa reward genuinely restorative tha?
- Kya woh safe, affordable aur non-compulsive tha?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 19 / 30 • WEEK 3

Consent Aur Conversation

Respectful choice ka foundation consent hai.

Aaj ek calm conversation ya rehearsal karo: facts, impact, responsibility, plan, requested support. No graphic detail unless clinically necessary. Other person ka no, pace and boundaries respect karo.

TRACK S • SUBSTANCE RECOVERY
Intoxication mein major relationship/sexual decisions avoid. If substance use has affected consent, safety, driving, shared money or health, relevant professional conversation priority.

TRACK M • MASTURBATION / PORN COMPULSION
Partner agreements specific and non-coercive. Monitoring/punishment/sex as reward nahi. Non-consensual or illegal risk ho toh specialist safety help; another person’s protection first.

EVENING REFLECTION

- Maine kya responsibility li?
- Kaunsi boundary clearly respect karni hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 20 / 30 • WEEK 3

Practical Repair

Life stability recovery ko support karti hai.

One practical repair choose karo: essential work task, study attendance, bill/budget, appointment, home responsibility. Full life one day mein fix nahi. Disclosure only necessary information to appropriate person.

TRACK S • SUBSTANCE RECOVERY
Shared money/substance-related debt/risk ho toh safeguards and professional/legal advice where needed. Withdrawal/intoxication mein major financial decisions avoid.

TRACK M • MASTURBATION / PORN COMPULSION
Paid content, lost work time, device/privacy harm, or relationship agreement repair ko concrete action do. No impulsive public confession or graphic disclosure as punishment.

EVENING REFLECTION

- Kaunsa repair further harm rokta hai?
- Next small practical step kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 21 / 30 • WEEK 3

Week Three Review

Support ko fit karo, life ko support ke peechhe nahi.

Review health, control, treatment, sleep, connection, work, relationships and meaning. Kya plan too restrictive, too vague, or unsupported hai? One KEEP / CHANGE / ASK plus any escalation.

TRACK S • SUBSTANCE RECOVERY
Repeated use/lapse, severe cravings, side effects, mental-health symptoms or unsafe home ho toh higher level of care discuss karo. More rules alone enough nahi.

TRACK M • MASTURBATION / PORN COMPULSION
Repeated loss of control, injury, distress, consent/relationship harm or obsessive rituals ho toh qualified CSBD/sexual-health/mental-health assessment strengthen karo.

EVENING REFLECTION

- Plan mein kya sustainable hai?
- Kahan professional/support level badhana chahiye?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 22 / 30 • WEEK 4

High-Risk Event Plan

Known transition ko pehle design karo.

Upcoming event/transition choose karo: travel, party, payday, deadline, alone weekend, conflict. Place, people, time, access, sleep and support map karo. Exit, backup and check-in written rakho.

TRACK S • SUBSTANCE RECOVERY
Substance-free alternatives, refusal, transport, no impaired driving, medication/treatment continuity and emergency plan. Travel medicine/legal requirements clinician/pharmacist se confirm karo.

TRACK M • MASTURBATION / PORN COMPULSION
Hotel/device privacy, idle time, social media, stress and consent boundaries plan karo. Device barrier plus support contact; blocker alone not enough.

EVENING REFLECTION

- Event ka highest-risk link kya hai?
- Exit/backup/check-in kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 23 / 30 • WEEK 4

Lapse Plan Drill

Setback se pehle safe response rehearse karo.

Imagined lapse/near-miss ke liye sequence rehearse karo without vivid detail: stop escalation → emergency check → safe person/place → professional contact → practical repair → analysis. No punishment or public shame.

TRACK S • SUBSTANCE RECOVERY
Reduced tolerance, overdose and medication interaction caution. No extra substance, random sedative, double/restarted medicine. Emergency signs 112; non-emergency lapse same-day/earliest clinical contact.

TRACK M • MASTURBATION / PORN COMPULSION
Shame-binge permission thought ko interrupt: device away, location change, support, ordinary task. Consent/health/financial harm ho toh relevant professional/repair response.

EVENING REFLECTION

- First safe action kya hai?
- Kisko aur kab contact karunga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 24 / 30 • WEEK 4

Shame Se Responsibility

Dignity aur accountability saath rakh sakte hain.

Ek difficult moment ko three lines mein rewrite karo: fact without insult, impact without minimising, next responsible action. ‘Main hopeless hoon’ ko ‘main support increase kar sakta hoon’ se replace karo.

TRACK S • SUBSTANCE RECOVERY
Return to use/lapse ko hide karna shame ka action ho sakta hai. Honest clinical disclosure safety improve karti hai. Treatment ko continue/revise karo; self-punishment nahi.

TRACK M • MASTURBATION / PORN COMPULSION
Normal desire ko disease mat banao; actual harm ko dismiss bhi mat karo. If another person affected, respect their boundary and repair without demanding forgiveness.

EVENING REFLECTION

- Fact / impact / action kya hai?
- Kis safe person ke saath honesty possible hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 25 / 30 • WEEK 4

Mental Health Check

Whole person ko treat karna recovery ka part hai.

Mood, anxiety, sleep, intrusive thoughts, trauma symptoms, energy/impulsivity, functioning and substance/behaviour link briefly review karo. Persistent/severe changes professional assessment deserve karte hain. Self-harm/suicidal thoughts, hallucinations, violence risk = urgent help.

TRACK S • SUBSTANCE RECOVERY
Substance/withdrawal and mental-health symptoms overlap kar sakte hain. Clinician ko full timeline do; self-diagnose ya medicine adjust mat karo. Tele-MANAS 14416 / 1800-891-4416; emergency 112.

TRACK M • MASTURBATION / PORN COMPULSION
Sudden sexual behaviour change, very low sleep with high energy/disinhibition, OCD-like intrusive thoughts, trauma response or severe distress ko specialist assessment do. Sirf willpower exercise enough nahi.

EVENING REFLECTION

- Kaunsa symptom review maangta hai?
- Main kaunsi help aur kab contact karunga?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 26 / 30 • WEEK 4

Support Boundaries

Support respectful aur sustainable ho.

Apne support agreements review karo: role, availability, privacy, backup, emergency escalation. Kya koi person overloaded hai? Kya monitoring coercive ban rahi hai? One boundary clarify karo.

TRACK S • SUBSTANCE RECOVERY
Family secret detox/medication confiscation nahi. Safety boundaries clear ho sakti hain: no use around children, no impaired driving, no abusive behaviour. Violence ho toh specialist safety plan.

TRACK M • MASTURBATION / PORN COMPULSION
Partner accountability consensual and minimally invasive. No forced passwords, body checks, graphic interrogation, sexual obligation or public exposure. Privacy with agreed accountability.

EVENING REFLECTION

- Kaunsi boundary support ko safer banayegi?
- Kya external monitoring kam/revise honi chahiye?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 27 / 30 • WEEK 4

Meaning Beyond Recovery

Life ko old pattern ke centre se hatao.

Ek non-recovery role/value choose karo: learner, friend, creator, parent, professional, community member. Uske liye one small action karo. Recovery important hai, but your whole identity nahi.

TRACK S • SUBSTANCE RECOVERY
Substance-free meaningful activity routine mein rakho. Early days mein big achievement pressure nahi; treatment stability ke saath one valued step enough.

TRACK M • MASTURBATION / PORN COMPULSION
Sexuality/relationship performance ko worth ka test mat banao. Non-sexual interests, affection, creativity and contribution build karo. Abstinence choose ho toh bhi life full ho sakti hai.

EVENING REFLECTION

- Kaunsa role mujhe meaningful lagta hai?
- Aaj ka one value-based action kya tha?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 28 / 30 • WEEK 4

Professional Plan Review

Treatment ko update karna strength hai.

Next appointment/review schedule karo ya confirm karo. One-page brief update: pattern, risks, medicines, symptoms, impacts, what worked, what did not. Five questions prepare karo.

TRACK S • SUBSTANCE RECOVERY
Medication goal, side effects, interactions, lapse/missed dose instructions, review and stopping/taper decisions prescriber se. Detox alone long-term plan nahi; continuing care discuss karo.

TRACK M • MASTURBATION / PORN COMPULSION
Therapist se case formulation, CBT/ACT/mindfulness/relapse-prevention approach, progress and limited evidence expectations discuss karo. No guaranteed cure or self-medication.

EVENING REFLECTION

- Mera main clinical question kya hai?
- Next review date aur contact kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 29 / 30 • WEEK 5

Green, Amber, Red

Early warning ko action se link karo.

Personal signs likho. Green: routine stable. Amber: isolation, sleep change, secrecy, risky contacts, bypassing barriers, missed support. Red: lapse with risk, severe symptoms, self-harm/violence/consent danger. Har zone ke actions likho.

TRACK S • SUBSTANCE RECOVERY
Amber par support/access/treatment review; red par emergency/professional response, no solo management. ‘Cured, support not needed’ ko possible amber sign treat karo if other signs present.

TRACK M • MASTURBATION / PORN COMPULSION
Amber par device/environment/support increase and therapist contact; red par health/consent/illegal/self-harm risk specialist/emergency. Ordinary lapse without red risk still needs prompt reset/review.

EVENING REFLECTION

- Mere top three amber signs kya hain?
- Red zone ka exact action sequence kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

DAY 30 / 30 • WEEK 5

Review Aur Next Thirty Days

Workbook khatam, support khatam nahi.

Month ka evidence review karo: health/safety, control, treatment, sleep, work/study, money, relationships/consent, mood/meaning. Keep three useful actions, change one weak plan, schedule next support. Missed days ko failure mat banao.

TRACK S • SUBSTANCE RECOVERY
Medication/treatment continue as prescribed; follow-up and lapse plan ready. Stable ho toh bhi support suddenly stop nahi. Repeated risk ho toh higher level of care discuss karo.

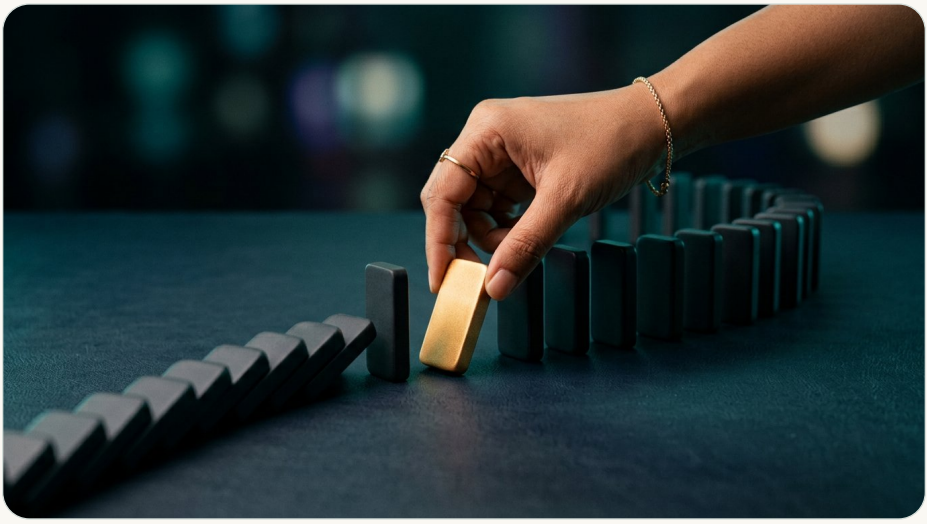
TRACK M • MASTURBATION / PORN COMPULSION
Frequency ke saath choice and functioning assess karo. Shame/obsessive counting badha ho toh tracking method revise. Therapy/support continue; next meaningful life goal choose karo.

EVENING REFLECTION

- Next month ke KEEP / CHANGE / ASK kya hain?
- Meri next review date, meaningful goal aur emergency plan kya hai?

Optional effort score: 0 not done • 1 attempted • 2 completed | Score effort, not worth.

TOOLKIT



Reusable Worksheets

Print, copy or recreate privately — no graphic detail needed

In templates ko repeat use karo. Agar writing anxiety ya compulsive checking badhati hai, toh shorter version choose karo. Safety information current aur accessible rakho; sensitive notes ko shared device/public place par unsecured mat chhodo.

REUSABLE TEMPLATE 01

Trigger Chain Map

Ek recent pattern ko earliest modifiable link tak trace karo.

01 Vulnerability: sleep, stress, conflict, health, withdrawal

02 Trigger: person / place / time / device / access

03 Permission thought

04 First action and immediate relief

05 Later cost

06 Earliest safe interruption + backup

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 02

Coping Card

High-pressure moment mein readable short plan.

01 My top warning sign

02 My first two-minute action

03 Location/access change

04 Two contacts and backup

05 Emergency signs → 112 / emergency care

06 Values-based reminder

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 03

Clinician One-Page Brief

Appointment mein safety-relevant facts clearly share karo.

01 Main concern and goal

02 Pattern / approximate amount or frequency / duration / last episode

03 All substances, medicines, allergies and conditions

04 Withdrawal, overdose, injury, mental-health or consent/safety risks

05 Previous attempts and treatment

06 Questions, next steps and review date

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 04

Lapse Functional Review

Shame ke bina escalation rokna aur plan revise karna.

01 Immediate safety addressed? Emergency/professional contact?

02 Vulnerability and trigger

03 Permission thought and access

04 Which barrier worked / failed / was bypassed?

05 Practical harm containment

06 One high-impact plan change + follow-up date

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 05

Weekly KEEP–CHANGE–ASK

Evidence se plan ko adjust karo.

01 Health and safety evidence

02 Control, treatment and functioning evidence

03 KEEP: most useful action

04 CHANGE: weakest/misfit plan element

05 ASK: specific support needed

06 Escalation or next appointment

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 06

Support Agreement

Help ko consensual, bounded aur sustainable banao.

01 Purpose of support

02 Role and what is NOT expected

03 Availability and backup

04 Privacy and information limits

05 Emergency response

06 Review date and either person’s right to revise

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 07

Relationship Repair Page

Responsibility, safety and consistent action.

01 Concrete impact acknowledged

02 Responsibility without blaming partner/stress/urge

03 Relevant health, consent or financial safeguards

04 Actions already taken and professional plan

05 Other person’s boundaries and pace

06 Next review without demanding forgiveness

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

REUSABLE TEMPLATE 08

Next Ninety Days

Workbook ke baad continuing rhythm.

01 Four weekly anchors: health, connection, responsibility, restoration

02 Known high-risk transitions

03 Green / amber / red actions

04 Professional and peer support schedule

05 One meaningful life goal beyond recovery

06 Emergency and lapse plan location

Safety remains primary
Worksheet ka answer emergency/professional care delay nahi karna chahiye. Jo prompt applicable nahi hai usse skip ya clinician ke saath adapt karo.

READER'S GUIDE

Glossary

Terms ko samajhna hai, khud ko label karna nahi.

Addiction / substance-use disorder

Substance use ka clinically significant pattern jismein impaired control aur harmful consequences central ho sakte hain. Severity aur diagnosis qualified assessment se.

Physical dependence

Body ka substance ke adjust hona, jiske reduction/stopping par withdrawal ho sakta hai. Yeh alone addiction ke barabar nahi; prescribed appropriate use mein bhi ho sakta hai.

Tolerance

Same effect ke liye more substance ki need ya same amount ka reduced effect. Period of non-use ke baad tolerance kam ho sakti hai, especially overdose risk ke liye relevant.

Withdrawal

Reduction/stopping ke baad emerging symptoms. Risk substance, amount, duration and health par depend karta hai; alcohol/sedative withdrawal dangerous ho sakta hai.

Craving / urge

Use/behaviour karne ki strong desire or pressure. Yeh experience ho sakta hai without action; severe medical symptoms ke saath craving ko simple urge na samjho.

Trigger / cue

Person, place, emotion, body state, time, device or access jo learned behaviour sequence ko activate kar sake.

Permission thought

Behaviour se pehle ka thought jo exception/justification deta hai, jaise 'bas ek baar' ya 'ab kya farak'.

Lapse

Return to use/behaviour ka episode. Catastrophise nahi, but safety and early review important.

Relapse

Problematic pattern mein broader return; terminology vary karti hai. Treatment ko resume/revise karna central response hai.

CSBD

Compulsive sexual behaviour disorder. ICD-11 impulse-control disorder category mein; persistent impaired control plus marked distress/impairment, with exclusions.

PPU

Problematic pornography use. Research/clinical shorthand; har porn use PPU nahi, aur self-label diagnosis nahi.

CBT

Cognitive behavioural therapy: situations, thoughts, emotions, behaviours and consequences ke patterns par structured work.

Motivational interviewing

Collaborative approach jo ambivalence explore karke person ke own reasons for change strengthen karti hai.

Contingency management

Specific healthy target behaviours ko structured positive reinforcement dena; stimulant-use disorder treatment mein strong evidence.

NRT

Nicotine replacement therapy. Appropriate cessation support; suitability, product, dose and duration qualified clinician se.

Continuing care

Initial detox/therapy/programme ke baad ongoing appointments, support, medication review and relapse prevention.

Co-occurring condition

Substance/sexual-behaviour concern ke saath another mental/physical health condition, jise integrated assessment/treatment chahiye ho sakta hai.

Consent

Freely given, informed, specific, reversible agreement with capacity. Silence, relationship status or intoxication automatic consent nahi.

Self-compassion

Difficulty acknowledge karke dignity maintain karna and responsible safe action lena; excuse ya accountability removal nahi.

Harm reduction / safety planning

Immediate risks kam karne ke practical steps, treatment ke against nahi. Emergency signs mein urgent professional care central hai.

READER'S GUIDE

Verified Help Directory — India

Official sources se checked 1 November 2026. Services/hours can change; verify when needed.

Immediate emergency — India

112

Seizure, unresponsiveness, breathing problem, severe confusion, serious injury, immediate self-harm/violence danger or other urgent police/fire/medical emergency.

Official source: R22

Tele-MANAS

14416 / 1800-891-4416

24×7 mental-health support, counselling and referral. Immediate physical danger/overdose mein emergency care priority.

Official source: R21

National Drug-de-Addiction Helpline

14446

Substance-use concern mein counselling, treatment/service navigation and NAPDDR-related support.

Official source: R20

National Tobacco Quitline

1800-11-2356

Tobacco cessation counselling and quitting support.

Official source: R19

Nearest qualified service

Ask / verify locally

Government hospital, medical college, de-addiction service, psychiatrist, physician, clinical psychologist, sexual-medicine professional. Credentials, humane care, medical support and aftercare verify karo.

Official source: R20

Agar helpline connect na ho
Immediate danger mein 112 / nearest emergency department. Non-emergency mein nearest government hospital, medical college, qualified physician/psychiatrist ya de-addiction service contact karo. Ek unavailable number ko help unavailable samajhkar give up mat karo.

Professional choose karte hue

- Credentials, relevant experience and humane treatment approach.
- Withdrawal/medical support, emergency protocol and referral pathway.
- Consent, privacy, family involvement and transparent charges.
- No violence, forced labour, public humiliation, unlawful confinement or miracle-cure guarantee.
- Aftercare and lapse plan included in discharge/continuing care.

READER'S GUIDE

Research Notes & References

Original Hinglish synthesis • 26 linked sources • Accessed 1 November 2026

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Har source ko read karke conclusions ko original practical framework mein synthesise kiya gaya hai. Yeh book kisi source ka copied translation ya replacement nahi. Clinical guidance ke liye current official documents and qualified professionals consult karo.

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Azaadi Ka Matlab Choice Ka Wapas Aana

Tumhe apne past behaviour ke liye lifelong shame carry nahi karna. Responsibility lo, safe help lo, aur next choice ko thoda clearer banao. Recovery perfect streak nahi — honest, supported, repeatable life-building hai.

EMERGENCY **112**
DRUG DE-ADDICTION **14446**
TELE-MANAS **14416 / 1800-891-4416**
TOBACCO QUITLINE **1800-11-2356**

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